

Medicare Made Plain

*The 2027 Enrollment Season Guide: Parts, Plans, and
Deadlines Explained Without the Jargon*

Walter Brennan

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The Tuesday Table

Every Tuesday morning I sit at the long table in the Mechanicsburg public library, under the window that leaks cold air in January, and somebody sits down across from me with a stack of envelopes.

It's always the same stack. A glossy one from an insurance company showing two people on bicycles. A thin one from Social Security. Three nearly identical booklets from plans the person has never heard of. A postcard with ACT NOW printed in red. And underneath all of it, still sealed, the gray-and-blue "Medicare & You" handbook that arrived in September.

Last fall a man in a Penn State windbreaker set his pile down, pushed it two inches toward me, and said, "Just tell me which one."

I didn't. I never do. It isn't modesty.

Two questions, and only you have the answers

Here is the part the television ads leave out. There is no best Medicare plan. There is only the plan that fits one particular person's doctors, prescriptions, travel habits and tolerance for a bad year.

Let me put it on the board.

1. Which doctors, hospitals and pharmacies do you need to keep?
2. How much money could you hand over in a year when something goes seriously wrong, without it wrecking you?

I can explain every rule in the program. I cannot answer those two questions for you, and until they're answered, every piece of advice anybody gives you is a guess wearing a suit.

The man in the windbreaker turned out to have a cardiologist in Hershey he'd been seeing for eleven years and a daughter in Oregon he visits for a month each summer. Those two facts narrowed his real choices faster than any brochure on the table. We spent most of the hour on his doctor list, not on premiums.

That's the whole method of this book. Answer the two questions, then let the rules tell you what's possible.

Why I taught this to myself

I taught civics and American government at a Cumberland County high school for 34 years. Thirty-four years of explaining to teenagers why a law says what it says, and why the sentence is so long.

Then I turned 65, my district's retiree health coverage ended, and I discovered that knowing how a bill becomes a law teaches you nothing about Part B.

Joan sat beside me at our kitchen table reading drug names off a pharmacy printout while I clicked through screens. We got through it. I also got stubborn about it, in the way retired teachers do, and read the handbook cover to cover, then the Social Security Administration's own explainers, then the handbook again. Neighbors started asking. The library table came after that.

I have no insurance license. I've never sold a policy, I don't get paid when you pick a plan, and I'd be useless to an insurance company because I keep telling people to check the plan's own documents instead of its commercials. My strong opinion, built one Tuesday at a time: afternoon television and brother-in-law advice cause more bad Medicare decisions than the rules ever have.

Why this book says 2026 when the cover says 2027

Before you price a 2027 plan, check which year's numbers are printed here.

Medicare's dollar amounts change every single year. The Part A and Part B premiums, the deductibles, the yearly cap on what you pay for prescriptions, the income thresholds that raise your premium if you earned above them. All of it is re-set annually by the Centers for Medicare and Medicaid Services, with the income-related amounts published by Social Security.

CMS and SSA publish the next year's amounts in the fall, but the release date varies. Check the official cost pages before you act on any number. I'm not going to promise you a date on an agency's behalf, and nobody else should either.

Some 2027 Part D amounts are already published and identified as 2027 in Chapters 4 and 9; where CMS or SSA has not yet posted 2027 amounts, this book uses clearly labeled 2026 figures. Check official figures again before enrolling.

Use Chapter 9 to replace the dated federal figures; then separately verify current plan documents, formularies, provider networks and state rules before choosing coverage. A penciled number does not update the rest of the book.

The rules themselves move far more slowly than the prices. A seven-month enrollment window is still seven months. The reason a late penalty exists hasn't changed since Congress wrote it. Learn the structure from these pages, then verify the dollars at the source.

The order things happen in

Medicare isn't one decision. It's a sequence, and the sequence has a brutal feature: some of its doors only open once.

So the chapters follow the order a real person meets these choices, not alphabetical order and not the order an insurance glossary would use.

First, what Parts A, B, C and D actually are, because almost every conversation at my table starts with a misunderstanding about one of the four. Then enrollment timing and the penalties for missing it, which is the chapter I'd read tonight if you're anywhere near 65. Then the big fork: Original Medicare with a Medigap policy, or a Medicare Advantage plan. Then prescription coverage and how to compare plans on total yearly cost rather than the monthly premium, which is where most of the money hides. Then Medigap and its one-time window. Then the income-related surcharge called IRMAA, which catches people by surprise because it looks back at a tax return from two years earlier. Then the help available at the other end of the income scale, which far more people qualify for than claim. Then working past 65, and the employer-coverage traps that come with it.

Every substantive chapter ends with a checklist, a worksheet or a decision rule you can carry to medicare.gov's Plan Finder or to a counselor. They're meant to be written on.

You can read this straight through in an evening or two. If you're short on time, read the enrollment chapter and the one on your own live question, then come back for the rest before the fall.

One boundary. People ask me at the same table when they should claim Social Security, and it's a different decision with different math, so it gets its own book: *Social Security, Claimed On Purpose*. Here I'll stay on Medicare.

Where to get help that isn't me

I can tell you how the rules work. I can't look at your situation, and neither can a book.

Three places can, and none of them earn a commission. Medicare's own line, 1-800-MEDICARE, is staffed year-round. Every state runs a State Health Insurance Assistance Program, usually called SHIP; in Pennsylvania ours is APPRISE. SHIP counselors offer free, unbiased one-on-one Medicare counseling; find your local program at shiphelp.org. And medicare.gov's Plan Finder will do arithmetic that no brochure wants you to do.

Call a licensed agent or a tax professional when the stakes outgrow a checklist. A premium surcharge landing the year after you retire is one of those moments. So is weighing an employer plan against Medicare with a spouse's coverage in the mix, or facing a health condition that could let a Medigap insurer turn you down. Pay someone who can look at your actual numbers. I'll flag those moments as we reach them.

A few rules also vary by state, and the three states that standardize Medigap differently will get named where it matters. When something depends on where you live, I'll say so rather than let you assume Pennsylvania's answer is yours.

That's the rule. You don't have to like it.

Before you turn the page

Go find the envelopes. The glossy ones, the booklet, the postcard with the red lettering, whatever came in the mail and got moved to the counter so you could eat dinner.

Put them in one pile, somewhere you'll see it. Then get a pencil, because the worksheets in here don't work in your head.

The man in the windbreaker kept his pile in a canvas tote bag hanging on the back of a kitchen chair. He said it annoyed him into dealing with it. Good enough.

Chapter 1: Four Letters, but Not Four Add-ons

I keep one sheet of paper in the folder I bring to the library. Four letters down the left margin, in pencil, one word beside each. A, hospital. B, doctor. C, delivery. D, drugs.

That sheet exists because of a question I get more than any other, usually phrased exactly this way: “So do I buy B, or do I buy C?”

Fair question. There’s a wrong idea buried in it, though, and the wrong idea is expensive.

Medicare has four parts, but they don’t sit in a row like items on a menu. Parts A and B make up Original Medicare. Part C is another way to receive those benefits, usually with Part D included. Part D covers outpatient prescription drugs. Until the shape is clear, every comparison you try to make is a comparison between things that aren’t comparable, and you’ll feel stupid for being confused when the fault is in the naming.

So. Letters.

A is the hospital part, and its deductible works oddly

Part A covers inpatient care in hospitals, skilled nursing facility care, hospice care, and home health care. That’s Medicare.gov’s own list of what it helps pay for, and the word carrying the weight is *inpatient*.

Most people pay no monthly premium for Part A. Medicare.gov says it’s \$0 for most people, because they or a spouse paid Medicare taxes long enough while working, generally at least ten years. If you

don't have that work record, you may be able to buy Part A, and in 2026 the buy-in premium is either \$311 or \$565 a month depending on how long you or your spouse paid in, according to Medicare.gov's cost pages. Those are 2026 amounts. Check the current one before you budget around it.

Premium-free doesn't mean free. Here's where people get hurt.

In 2026, Medicare.gov puts the Part A deductible at \$1,736, and you pay it for each inpatient hospital benefit period, not once a year. Read that sentence twice. Your Part B deductible is annual, your Part A deductible is per benefit period, and Medicare.gov states plainly that there's no limit to the number of benefit periods you can have in a year. Two separate hospital admissions far enough apart, and you can pay that deductible twice in the same calendar year.

How the clock on a benefit period starts and stops is its own small piece of arithmetic, and Medicare.gov has a page explaining it, linked right from the cost table. Look it up when it matters to you. What you need in your head now is just this: Part A's big number can repeat.

After the deductible, Part A's 2026 inpatient hospital schedule runs like this, per Medicare.gov. Days 1 through 60 cost you nothing more. Days 61 through 90 cost \$434 each day. Days 91 through 150 cost \$868 each day, while you spend down the 60 lifetime reserve days you get once in your life. After day 150, you pay everything.

Skilled nursing care has its own ladder. In 2026, Medicare.gov lists days 1 through 20 at \$0 after the Part A deductible, days 21 through 100 at \$217 each day, and everything after day 100 on you. Part A limits that coverage to 100 days per benefit period.

And one trap worth a paragraph of its own, because it has blindsided people I've sat with. Medicare generally covers a skilled nursing stay only after a qualifying inpatient hospital stay of at least three days in a row. Medicare.gov says time spent under observation or in the emergency room before you're admitted doesn't count

toward those three days, even if you were there overnight. You can sleep in a hospital bed for two nights, be billed as an outpatient, and fail the test. If you or a parent is in a hospital and somebody mentions rehab afterward, ask the question out loud: am I admitted as an inpatient, or under observation? Also ask the hospital and your plan whether an approved ACO or Medicare Advantage three-day-stay waiver applies; other SNF eligibility requirements still matter. Write down the answer and the date.

Hospice is the gentlest part of Part A's cost structure. For 2026, Medicare.gov lists \$0 for covered hospice services, up to \$5 per qualifying home pain-or-symptom prescription, and 5% of the approved amount for inpatient respite; verify the coverage-year amounts before relying on them.

The bed does not decide which part pays

A hospital wristband tells you where you are. It does not establish inpatient coverage. That difference matters before a bill arrives because the admission status can affect the next place you receive care.

On Medicare.gov's [inpatient and outpatient status page](#), inpatient status begins with formal admission under a doctor's order. Until that happens, emergency department care and observation are outpatient services, even if you stay overnight. The page tells you or your caregiver to ask about your status each day. Do that while the hospital can still explain the order, not from the kitchen six weeks later.

The official page supplies a useful contrast: an emergency visit followed by a formal admission is not the same as outpatient surgery followed by an overnight stay without an admission order. The first can bring an inpatient hospital benefit into the picture. The second

remains outpatient. How ill you felt and how many hours you spent waiting cannot settle that distinction on their own.

If observation lasts more than 24 hours, the hospital must provide a Medicare Outpatient Observation Notice, called the MOON, according to Medicare.gov. It explains the observation status and its possible cost consequences. The initials sound like something a retired science teacher would explain. Here they mean you should read the page before agreeing that you understand it. Ask what status applies now and whether it changed during the stay.

There can also be two parts paying for one inpatient episode. Medicare.gov's inpatient-care page says Part B generally covers the doctor's services during a hospital stay. Part A's hospital schedule is therefore not a price for every professional service connected with that admission. When estimating a planned stay, ask the hospital which charges are facility charges and which may arrive from clinicians. Do not assume a hospital deductible absorbs a separate physician bill.

For the skilled nursing question introduced above, the discharge day does not count toward the qualifying three-day inpatient stay, Medicare.gov says. Nor does observation time before admission. That makes the admission order's date important. Ask the discharge planner to explain how the actual dates satisfy the rule or how an approved waiver applies; do not count nights yourself and declare the question settled.

The same SNF coverage page requires more than those dates. It names remaining benefit-period days, admission to the SNF generally within 30 days after leaving the hospital, daily skilled-care need, and a Medicare-certified facility. The required care must relate to a condition treated during the qualifying stay or a new condition arising while receiving SNF care for that condition. These are eligibility conditions, not a promise that a reserved bed comes with Medicare payment.

Skilled care can be needed to improve a condition, maintain it, or prevent or slow deterioration, the page explains. Do not translate “rehab” into “coverage only while improvement is visible.” The treating team must assess the skilled need. Conversely, needing a place to recover or help with everyday tasks does not by itself establish the Medicare skilled-care benefit. Have the team identify the benefit being requested and who will provide the qualifying care.

Home health does not mean someone stays all day

The words *home health* can make a household imagine a much larger service than the Medicare benefit provides. A nurse visiting to manage a wound is a different arrangement from someone remaining in the house whenever a spouse goes out.

Medicare.gov’s [home health coverage page](#) ties eligibility to being homebound and needing part-time or intermittent skilled services. Its homebound definition concerns difficulty leaving because of illness or injury, the help needed to leave, and the considerable effort involved. It does not mean you are forbidden to leave for medical treatment or short, infrequent non-medical absences. The page also says adult day care attendance does not automatically prevent home health care.

A clinician must see you in person, confirm the need, and order the care; a Medicare-certified agency must provide it. Covered services can include skilled nursing and qualifying therapies. Home health aide help is tied to receiving the specified skilled services, not an independent promise of ongoing bathing or housekeeping assistance for anyone with Medicare.

The official exclusions are the boundary a family should see before making a schedule. Medicare does not pay under this benefit for round-the-clock home care, delivered meals, shopping or cleaning

unrelated to the care plan, or personal care when that is the only care needed. If the discharge plan assumes someone will help at night, ask who that person is and how the service will be paid for. An agency's visit schedule cannot stand in for the hours no visitor is there.

Before home care begins, the agency should explain what Medicare is expected to pay and identify services it expects you to pay for, verbally and in writing. This is useful information for planning, not a verdict that every proposed service qualifies. An Advantage enrollee should also confirm the plan's home health terms. Write the actual ordered visits beside the household's uncovered hours; keep the medical care plan separate from the family's assumptions about supervision.

B is the one that sends you a bill every month

Part B is doctors and everything that happens while you're upright. Medicare.gov's list: services from doctors and other providers, outpatient care, home health care, durable medical equipment like wheelchairs and walkers, and many preventive services such as screenings, shots and the yearly wellness visit.

Part B costs money every month whether you use it or not. The 2026 standard premium is \$202.90 a month, per Medicare.gov, and it can be higher depending on your income, which is a whole chapter of its own later on. The 2026 Part B deductible is \$283 a year, once a year, and after you've met it you generally pay 20% of the cost of each covered service.

That 20% has no ceiling, and this is the part of Chapter 1 I'd underline if the ink were mine. Medicare.gov says it directly: there's no yearly limit on what you pay out-of-pocket in Original Medicare unless you have supplemental coverage, like a Medigap policy, or you join a Medicare Advantage plan.

Twenty percent of a knee replacement is not twenty percent of an office visit. Twenty percent of a year with cancer treatment in it is a number nobody can tell you in advance, because the only cap is the one you buy yourself. Remember the second of the two questions from the front of this book, the one about what a bad year could cost you? This is the sentence that makes that question real rather than philosophical.

A few Part B items cost you nothing extra. Medicare.gov lists \$0 for covered clinical laboratory services and \$0 for covered home health care services, while durable medical equipment runs at the usual 20% of the Medicare-approved amount.

One more thing about those percentages. They assume your doctor accepts the Medicare-approved amount as full payment, which Medicare.gov calls accepting assignment. Most do. Before an expensive procedure with a provider you haven't used before, it's a reasonable thing to ask the billing office in plain words.

Ask for the appointment by its covered name

"My annual physical" and "my Medicare wellness visit" sound interchangeable across a reception desk. They are not. Medicare.gov describes the [yearly wellness visit](#) as a conversation-based visit to create or update a prevention plan, not a routine physical examination.

The visit includes a health-risk questionnaire, review of medical and family history and current prescriptions, routine measurements, and a written prevention plan. That is useful work. It is not a promise that every test or new health concern raised during the appointment belongs to the same preventive benefit.

The first yearly wellness visit cannot take place within 12 months of Part B enrollment or a Welcome to Medicare preventive visit, the official page says; afterward, the frequency is once every 12 months.

The two preventive visits are different benefits. Ask the office which one it is scheduling and whether your previous visit date permits it. A new calendar year alone does not answer a rolling twelve-month question.

In the coverage guidance checked for 2026, the covered wellness visit has no patient charge when the provider accepts assignment, and the Part B deductible does not apply to that preventive benefit. Other tests or services during the appointment may bring deductible or coinsurance charges; something Medicare does not cover, such as a routine physical, may be entirely your responsibility. Ask about those separate services, without withholding symptoms the clinician needs to hear. Medical attention matters more than keeping a visit inside one billing label.

Transport has a similarly important boundary. Part B's [ambulance coverage page](#) requires medical necessity and a situation in which another vehicle could endanger your health. Coverage is to the nearest appropriate facility able to provide the needed care, not automatically to a preferred hospital farther away. Certain non-emergency trips may qualify with the required provider order; being unable to find a ride does not establish ambulance coverage by itself.

For a scheduled non-emergency ambulance trip, ask the ordering clinician and transport company about the coverage conditions and any approval needed. For a medical emergency, get emergency help instead of trying to price the ride first. The coverage question belongs in advance when the trip can safely be planned, not between a frightened person and urgent treatment.

C isn't a third kind of coverage

Here's the correction I make most often at that table.

You do not buy Part C in addition to Parts A and B. Medicare Advantage, which is the friendlier name for Part C, is an alternative

way to get your Part A and Part B coverage, through a private plan that Medicare has approved. Medicare.gov describes these as bundled plans that include Part A, Part B, and usually Part D.

On the board, then. One, you keep Medicare. Two, a private company administers it for you, under Medicare's rules. Three, you still pay your Part B premium, and you must keep paying it to stay in the plan, which Medicare.gov states on its cost pages.

What you give and what you get both change inside that arrangement. Medicare.gov says you may need to use doctors in the plan's network, and get prior authorization for certain drugs or services before you receive them. It also says out-of-pocket costs usually differ from Original Medicare and include a yearly limit on what you pay, and that most plans offer extra benefits Original Medicare doesn't cover, such as vision, hearing and dental.

A yearly out-of-pocket limit. After a section that just told you Original Medicare has none, you can see why that sentence sells plans. It's a real advantage, and it isn't the whole story, and the trade involves networks and permissions rather than only dollars. That comparison is Chapter 3's entire job, and I'm not going to half-do it here.

The sorting rule for now: Original Medicare plus optional add-ons is one road. A Medicare Advantage plan is the other road. You are not assembling both.

D is drugs, and you pick the plan yourself

Part D helps cover prescription drugs, and Medicare.gov is clear about how you get it: you join a Medicare drug plan alongside Original Medicare, or you get drug coverage by joining a Medicare Advantage plan that includes it. Either way, the plans are run by private insurance companies following rules Medicare sets.

Most people choose and join their own Part D plan, although Medicare automatically enrolls some people who qualify for Extra Help if they do not already have drug coverage. So if you get Extra Help with drug costs, check what plan your name is already sitting in before you shop. Part A and Part B are government coverage with government numbers. Part D is a private plan, from a list that looks identical until you put your own prescriptions into it, and the formulary — the plan's list of covered drugs — is where the real difference lives. Part D also has a yearly ceiling on what you pay out of pocket for covered drugs, which is a genuinely useful piece of recent design. Chapter 4 does the arithmetic and names the current cap.

The one sentence I want you carrying out of this section: a plan with the lowest premium on the page is not the same thing as the plan that costs you the least this year.

The fifth thing, which isn't a letter

Medigap belongs in this chapter even though it has no letter of its own, because people confuse it with Part C constantly.

Medigap is extra insurance you buy from a private company to help pay your share of costs in Original Medicare — that uncapped 20%, those repeating deductibles. Medicare.gov says these policies are standardized and, in most states, named by letters like Plan G or Plan K, and that the benefits in each lettered plan are the same no matter which insurance company sells it.

Sit with that last part, because it's unusual and it's useful. Two companies selling the same letter in the same state are selling the same benefits. Which means the thing left to compare is price and service, not coverage. Three states run their standardization differently, and I'll name them in Chapter 5, where Medigap gets the

treatment it deserves, including a once-in-a-lifetime window you should know exists before you need it.

Medigap works with Original Medicare. It does not work with a Medicare Advantage plan. That single incompatibility is why Chapter 3's fork matters more than any other decision in this book.

Which parts do you actually have

Now the practical bit. A surprising number of people don't know what they're holding, which makes every further question unanswerable. Take your red, white and blue Medicare card, your plan cards if you have any, and go down this list with a pencil.

1. Does your Medicare card show HOSPITAL (PART A), and what date does it say coverage started?
2. Does it show MEDICAL (PART B), with a start date?
3. Do you have a separate card from a private insurance company that says Medicare Advantage, HMO, PPO or Part C anywhere on it?
4. If yes to 3, does that same card mention drug coverage, Rx, or Part D?
5. If no to 3, do you have a separate drug plan card from a private insurer?
6. Do you have a Medigap or Medicare Supplement policy, and if so, which letter is printed on it?
7. Is anyone taking a Part B premium out of your Social Security payment, or are you being billed directly?
8. Write down every drug you take, the dose, and the pharmacy you use. You'll need this list twice before this book is finished.
9. Write down every doctor and hospital you refuse to lose. Same reason.

If you answered yes to item 3 and yes to item 6, call the company that sold you the Medigap policy this week. Those two pieces don't work together, and you may be paying a premium for something that cannot pay a claim.

If item 7 confuses you, or if you're still working and someone has told you to skip Part B, that's a question for a professional rather than a book. A licensed agent or a free counselor can see your actual paperwork, which I can't, and the cost of getting the working-past-65 rules wrong lands on you for years. Chapter 8 lays out the questions to ask them.

What the letters don't tell you

Four letters, maybe a dozen numbers, and a deductible that can arrive twice in one bad winter. That's the machinery.

None of it tells you which road to take. That's still the two questions, and they're still yours.

The next chapter is the one with a clock in it. Everything in this chapter stays true whether you read it today or in March. The enrollment windows do not wait, and missing one attaches a surcharge to your premium that can follow you for as long as you have the coverage.

Keep the pencil. Go find your card.

Chapter 2: Seven Months, and the Penalty That Doesn't Expire

Lorraine brings lemon bars to the library on Tuesdays and asks the hardest questions at the table. Two Octobers ago she brought a letter instead.

She wanted me to tell her the number on it was wrong. It wasn't.

Her husband had died the year she turned 64. His former employer's retiree plan kept covering her, and it was decent coverage, and so when her birthday came she did the sensible-sounding thing and left it alone. She already had insurance. Why buy Part B on top of it.

Because retiree coverage is not coverage from current work. One word, *current*, carries that entire distinction, and nobody in any office had said it to her.

Medicare.gov lists this exact situation among the ones that do not qualify for a Special Enrollment Period: your COBRA coverage or retiree coverage ends. So she signed up later, and a late enrollment penalty attached itself to her monthly premium, and it is still there.

She pays more each month than the neighbor who signed up on time, for identical Part B coverage. I told her so in those words, because softening it would only have cost her the chance to plan around it. Then we spent the rest of the hour on what could still be protected.

A clock runs through everything in this chapter. The rules in Chapter 1 hold whether you read them today or next March. These don't.

The seven months you were given without being told

Your first chance to sign up is called the Initial Enrollment Period. Medicare.gov describes it as lasting seven months, starting three months before the month you turn 65 and ending three months after that month.

Count it out on a calendar rather than in your head. If you turn 65 on June 2 through June 30, your seven months run March through September. If your birthday is June 1, Medicare treats May as your first eligibility month; confirm your exact seven-month window with Social Security's enrollment calculator before marking it.

Nobody mails you a reminder that your wing is closing.

Now the part almost everyone misses, and it costs real coverage rather than real money. Within those seven months, the date your coverage starts depends on which month you signed up in, and coverage always begins on the first of a month.

Three lines on the board.

1. If you qualify for premium-free Part A, Medicare.gov says that coverage starts the month you turn 65. If your birthday falls on the first of the month, it starts the month before you turn 65.
2. For Part B, and for Part A if you have to pay a premium for it, signing up *before* the month you turn 65 means coverage starts the month you turn 65.
3. Signing up during the month you turn 65, or in any of the three months after, means coverage starts the following month.

Read line 3 again. If you sign up for Part B in your birthday month, Part B generally starts the next month; it will not cover Part B services before its effective date. Part A may have a different effective date. Check your card and any other coverage before scheduling non-emergency care. Sign up in the third month after instead and that

gap is four months long. The window stayed open, you used it, you are not penalized, and the calendar still costs you.

So the instruction is simple and it is the single most useful sentence in this chapter: if you want Medicare coverage in place the month you turn 65, sign up during the three months *before* your birthday month. Front of the window, not the back.

Premium-free Part A plays by different rules

Part A behaves differently if you qualify for it at no premium, and the difference is generous enough to be worth knowing.

Medicare.gov says you can sign up for premium-free Part A any time after you turn 65. Your Part A coverage then starts six months back from when you sign up or apply for Social Security benefits, though never earlier than the month you turned 65. A retroactive start date, in a program with no other mercies of that kind.

That does not extend to Part B. It does not extend to Part A if you have to buy it. And if you have a Health Savings Account, a retroactive Part A start date can create a problem with the contributions you made during those months, which is a conversation for Chapter 8 where the working-past-65 decisions live.

Part B, once your seven months close, is only available during one of the other enrollment periods. There are three kinds, and they are not interchangeable.

The eight months that depend on one word

If you are 65 or older and still covered by health insurance through your own current job or your spouse's current job, you may have a Special Enrollment Period instead.

Medicare.gov describes this one carefully, and so will I. It starts the first month after your Initial Enrollment Period ends. It ends

eight months after the group health plan coverage ends or the employment ends, whichever happens first. Coverage generally begins the first month after you sign up. And if you sign up while you or your spouse are still working, or within the first full month after the employer coverage ends, you can ask to delay your Part B start date by up to three months.

Four things about that paragraph deserve their own breath.

Whichever happens first. Retire in May with the plan paying claims through August, and your eight months may already be running from May. People lose this window while believing it hasn't opened yet.

Current. Lorraine's word. That same Medicare.gov list of disqualifying situations also names COBRA coverage ending, Marketplace coverage, and missing the eight-month window itself. COBRA is the cruelest of those. The envelope arrives looking exactly like a continuation of employer coverage, and for Medicare's purposes it is not current employment coverage at all.

Disability situations differ. Medicare.gov notes that if you have a disability and your coverage comes from a non-spouse family member's current employment, that employer must be in a large group health plan for you to be eligible.

End-Stage Renal Disease is carved out. Medicare.gov states that this Special Enrollment Period does not apply if you are eligible for Medicare based on ESRD. If that is your situation, the sequence is different and this is one of the places to call 1-800-MEDICARE rather than trust a general rule.

If you are reading this while still working and someone has told you confidently to skip Part B, stop and verify it against your own employer's plan documents and your own employer's size. Not against your brother-in-law, and not against me.

January, March, and the window of last resort

Miss everything above and there is still a door, which Medicare.gov calls the General Enrollment Period: you can sign up between January 1 and March 31 each year, and your coverage starts the month after you sign up. Medicare.gov adds that you might pay a monthly late enrollment penalty if you don't qualify for a Special Enrollment Period.

Which means a person who realizes in June that they never signed up for Part B is looking at the following January at the earliest, with coverage starting in February or later, and a surcharge riding on top of the premium after that.

That can mean seven months without Part B coverage, followed by a penalty generally charged for as long as you have Part B. Nobody designed that outcome on purpose. It is still the outcome.

One naming problem to clear up while we're here, because it produces more confusion at my table than any other pair of terms. The General Enrollment Period, January through March, is for people who don't have Part B yet. The fall window you hear about in television ads, October 15 through December 7, is a different thing entirely, for people who already have Medicare and want to change plans. Chapter 3 handles that one. If somebody tells you in November that you can still sign up for Part B, ask them which window they mean, and then check it yourself.

What a penalty actually does to the number on your bill

Penalties in Medicare are not late fees. Medicare.gov puts it in four blunt lines. They are added to your monthly premium. They are not a one-time late fee. They are usually charged for as long as you have

that type of coverage, which for most people means a lifetime penalty. And they go up the longer you wait.

Part B's arithmetic, per Medicare.gov: you pay an extra 10% for each full 12-month period you could have signed up for Part B and didn't. Their own worked example uses two full years. Twenty-four months late means a 20% penalty. In 2026 that is \$40.58 added to the standard \$202.90 premium from Chapter 1, for a monthly premium Medicare.gov calculates at \$243.48 and rounds to \$243.50. Those are 2026 figures, and both halves of the sum move when the premium moves.

Forty dollars a month does not sound like a catastrophe. Multiply this year's monthly penalty by 12 for a one-year estimate, then recalculate when the standard premium changes.

Premium-Part A works differently, and this is the one piece of good news in the section. Medicare.gov says that if you have to buy Part A and don't buy it when you're first eligible, your monthly premium may go up 10%, and you pay that penalty for twice the number of years you didn't sign up. Their example: eligible for two years without signing up, so a higher premium for four years. It ends. Part B's generally does not.

There is a third penalty, attached to Part D, with its own percentage and its own 63-day rule about going without drug coverage. It lives in Chapter 4 alongside the drug plans, because the only sensible way to understand it is next to the thing it applies to.

Medicare.gov also notes that you generally avoid the Part B penalty if you qualify for a Special Enrollment Period or enroll in a Medicare Savings Program, and avoid the Part D penalty if you have creditable drug coverage or qualify for Extra Help. Those last two are Chapter 7, and they are worth looking at even if you assume your income disqualifies you.

That's the rule. Nobody is asking you to like it.

The windows nobody mentions

The seven months get talked about. What gets far less attention is that Medicare.gov lists a set of other Special Enrollment Periods for Part A and Part B, each with its own clock, several of them created for situations that were plainly unfair before. Here is what Medicare.gov says about each, and the clocks are not the same length.

1. Lost Medicaid coverage on or after January 1, 2023: the window runs from the date you are notified the coverage is ending through six months after it ends. Coverage starts the first of the month after you sign up, or you can ask for it to start back when the Medicaid coverage ended.
2. Missed a chance to sign up because of a declared disaster or emergency: the window starts on the day the federal, state, or local government declares it, or on the date stated in the declaration, whichever is earlier. It ends six months after the latest of the original declaration's end date, the last extension's end date, or the date the government revokes or announces the end of the incident period. Your coverage starts the first of the next month.
3. Released from incarceration on or after January 1, 2023: the window runs from the date of release through the twelve months after the month of release. Coverage starts the first of the month after you sign up, or you may request a start as early as the release date, subject to Medicare's limit of no more than six months of retroactive coverage.
4. Volunteering in a foreign country: the window starts the first day of the month in which volunteering ends, the organization loses its tax-exempt status, or your qualifying overseas coverage ends, and runs six months from that day. Coverage starts the first of the month after you sign up.

5. Having TRICARE: the window starts when Social Security notifies you that you missed your chance to enroll and ends twelve months later. Coverage starts the first of the month after you sign up, or you can ask for it to start back to when your Initial Enrollment Period ended.
6. Inaccurate or misleading information from your health plan or your employer, on or after January 1, 2023: this is the one that matters enormously to anyone in Lorraine's position going forward. Medicare.gov says the window starts the day you notify the Social Security Administration that your plan or employer misrepresented or gave you incorrect information, and ends six months later.

There is also a Special Enrollment Period for other exceptional conditions, which Medicare.gov says lasts at least six months and requires you to contact Social Security and ask for it. In every one of these, the decision is Social Security's, not the plan's and not yours. Ask them for an eligibility decision rather than assuming one.

For several of these, Medicare.gov directs you to form CMS-10797, completed and sent to your local Social Security office by fax or mail.

I am not going to tell you whether your particular story qualifies, because that determination belongs to Social Security and the details matter more than a book can carry. What I will tell you is that if a benefits administrator or an insurance agent told you something in writing that turned out to be wrong, you have a reason to make a phone call rather than a reason to be resigned. Bring the writing. Dates help.

Which window am I in

Take a pencil and answer these in order. Stop at the first one that fits you.

1. What is your 65th birthday? Write the month and year.
2. Write the three months before that birthday month, the birthday month, and the three months after. That block is your Initial Enrollment Period. Circle all three months before it: enrolling in any of them puts Part B coverage in place in your birthday month, but do it as early as you can.
3. Are you 65 or older with coverage based on current employment, yours or your spouse's? If yes, write the expected end date of both the employment and the coverage. The earlier of those two dates starts your eight-month clock. Do not calculate the final day yourself from this sentence; enter the actual end dates in Medicare's eligibility tool or have Social Security confirm the last enrollment day in writing.
4. Is your coverage retiree coverage, COBRA, or a Marketplace plan? Those coverages do not by themselves create the working SEP. Check whether you qualify through current employment or one of Medicare's other listed special situations before relying on the General Enrollment Period.
5. Have your seven months already closed with no Part B? Write down next January 1 and next March 31. That is the General Enrollment Period, with coverage starting the month after you sign up.
6. Did any of these happen to you: lost Medicaid, a declared disaster, release from incarceration, foreign volunteer service, TRICARE, or wrong information from a plan or employer? If yes, call Social Security before you assume you have to wait for January, and ask about form CMS-10797.

7. Do you have a Health Savings Account you're still contributing to? Flag it and read Chapter 8 before you sign up for anything, including premium-free Part A.
8. Have you already enrolled? Find the start date on your card and write it next to item 1, because every later chapter asks you for it.

If items 3 and 4 both seem to describe you, that means somebody has given you conflicting information about your own coverage, and it is exactly the situation to take to a free SHIP counselor or to the employer's benefits administrator in writing. Ask one question: is this coverage based on current employment, yes or no. Get the answer in an email.

Turn the window into an application

Knowing your enrollment period still leaves a practical question: where does the application go? An Advantage advertisement can put a telephone number directly in front of you without making that number the place to enroll in Part B.

Medicare.gov's [Ready to sign up for Part A and Part B](#) page directs enrollment in those parts to Social Security. Its links differ according to what you already have and why you are applying. Choose your actual situation before opening a form. Buying a private plan is not a substitute for completing the underlying Medicare enrollment.

For a person signing up at 65 for both parts, or only Part A, the page links to Social Security's online Medicare application. Use that official entry point rather than a sponsored search result. If job-based insurance is ending and you already have Part A, it provides an online route for adding Part B. It also directs the employer to complete CMS-L564, the employment-information form. Submit that evidence to the local SSA office using its postal or fax instructions.

That employer form supplies evidence. It does not itself request every piece of coverage you need. Keep the enrollment application and the employment evidence together so you can tell Social Security what you submitted. If an employer cannot complete its section, Medicare.gov gives an alternative: complete Section B as well as you can without signing it, and provide proof of job-based health insurance. Open the official page's proof instructions or ask Social Security which records it accepts; do not fabricate an employer signature to finish a packet.

The exceptional-conditions route described earlier uses CMS-10797. Medicare.gov directs that completed application to the local Social Security office. For a Part-A enrollee adding Part B during the General Enrollment Period, its separate instructions name CMS-40B. These form numbers are not interchangeable ways to make the same request. A form downloaded from an old folder may describe a different eligibility route from yours.

For railroad work, the official enrollment page points to the Railroad Retirement Board instead. That is a useful exception to the direction to call Social Security. If you or a spouse worked for a railroad, ask the RRB which office handles your enrollment before sending the packet to an office chosen by habit.

After filing, keep the acknowledgment distinct from the coverage decision. A saved submission screen tells you what you sent; it is not the Medicare card's effective-date entry. Medicare.gov says the welcome package includes that card and your Medicare Number, which you need for a Medicare account, private-plan enrollment or supplemental coverage. When it arrives, compare the Part A and Part B entries with the start dates you requested and expected.

If they do not agree, take the discrepancy to the enrollment agency promptly. State the application route, submission date and requested start month. Ask what is still needed and what date its record currently shows. This is a concrete follow-up question, unlike

“Did I do Medicare right?” Do not cancel existing insurance on the strength of a screen confirming that an application was received; coordinate the actual effective dates with the agency and current insurer.

Three dates on the calendar

Lorraine still comes on Tuesdays. These days she mostly asks questions on behalf of other people, which is its own kind of answer.

Go get a calendar, the paper kind if you have one, and write three things on it. The first of the three months before your 65th birthday month. January 1 of next year. And if you’re working past 65, the date your employment or your employer coverage is expected to end, whichever comes first.

That’s the whole homework. The rules in the next chapter cost money if you choose wrong. These cost money if you simply don’t look.

Chapter 3: Two Roads, and the One Your Doctors Are On

Danny called me from a parking lot in Florida on a Thursday morning in February.

He'd gone to an urgent care the night before with a tightness in his chest. Probably nothing, they told him, but see a cardiologist this week. So that morning he called the office a neighbor had recommended, gave them his information, and got told they weren't in his plan.

"I have Medicare," he said. "I'm holding the card."

He did have Medicare. He also had a Medicare Advantage plan sold in central Pennsylvania, where my brother spends seven months of the year, and a set of Florida doctors he wanted for the other five. Danny believes television. The ad had promised dental, a gym membership, and no monthly premium, and every one of those promises was true, and not one of them was the thing that mattered in that parking lot.

He got seen. It took longer to arrange than a week and cost him more than it would have at home. What it really cost him was a February spent on the phone instead of on a dock.

This is the fork in the road, and the rest of the book arranges itself around it.

Two columns

Original Medicare and Medicare Advantage are two ways of receiving the same underlying coverage, and their trade-offs run in almost

exactly opposite directions. That's why comparing them on one number, any one number, gets people hurt.

Two columns, then.

Left column, Original Medicare. Medicare.gov says you can use any doctor or hospital that takes Medicare, anywhere in the U.S., and the footnote spells out what "anywhere" means: the 50 states, the District of Columbia, Puerto Rico, the U.S. Virgin Islands, Guam, the Northern Mariana Islands, and American Samoa. No plan network. In most cases, Medicare.gov says, you don't need a referral to see a specialist, and in most cases you don't need approval in advance for Medicare to cover a service or supply.

Right column, Medicare Advantage. Per Medicare.gov, non-emergency care may have to come from providers inside the plan's network and service area. Some plans do cover non-emergency care out of network, and it may cost more. A referral may be required to see a specialist. And the plan may require prior authorization before it will cover certain services or supplies.

Now turn to money, where the columns reverse.

Original Medicare's cost shape you already know from Chapter 1: a monthly Part B premium, usually 20% coinsurance after the deductible, and no yearly ceiling on your share unless you hold supplemental coverage. Medicare.gov names what counts as supplemental: a Medigap policy, or other insurance such as Medicaid, employer, retiree, or union coverage.

A Medicare Advantage plan, per Medicare.gov, has out-of-pocket costs that vary from plan to plan, and a yearly limit on what you pay for covered Medicare services. Reach that limit and you pay nothing for covered services the rest of the year. Read the next part twice: Medicare.gov says that limit may be different for in-network and out-of-network services. One plan, potentially two ceilings, and only one of them applies if you're seeing someone outside the network.

You keep paying the Part B premium on either road. Medicare.gov adds that with an Advantage plan you may also pay the plan's own premium, that some plans have a \$0 premium, and that some may help pay all or part of your Part B premium.

One last line in each column, and it's the line nobody learns until they want to move. Medicare.gov says that in Original Medicare you can choose to buy Medigap to help pay the out-of-pocket costs Medicare doesn't cover. With a Medicare Advantage plan, you can't buy Medigap to cover your out-of-pocket costs. When you may buy a Medigap policy, and what happens if you try later, is Chapter 5's whole subject. Do not assume the door you walked through stays propped open behind you.

The third column people forget to draw

Most of what I've just described compares an Advantage plan with bare Original Medicare. That comparison is incomplete because supplemental coverage changes Original Medicare's cost exposure.

So draw the third column: Original Medicare with the Medigap policy introduced in Chapter 1, which per Medicare.gov helps pay the copayments, coinsurance and deductibles Original Medicare leaves to you. Money-wise, that means a monthly premium to the insurer, and Medicare.gov is explicit that your monthly Part B premium continues on top of it, that Medicare pays none of the policy premium, and that such premiums typically increase each year. Medicare.gov also says policies sold after 2005 carry no prescription drug coverage, so a separate Part D plan is usually a third bill.

Which letter covers how much of that cost sharing, and the one-time window to buy without being asked about your health, is Chapter 5's subject. What matters here is the shape. The Original-plus-Medigap column trades a higher, more predictable monthly bill for less exposure in a bad month. The Advantage column trades a

lower monthly bill for a network, a permission step, and a yearly limit you may have to reach the hard way.

You cannot compare the two honestly without a real Medigap number. Medicare.gov lets you compare policy costs in your area, and tells you to contact the insurance company for a more accurate price, because the site's figures are estimates. Get that quote before you fill in the worksheet at the end of this chapter.

The two words that cost Danny his February

Network and service area.

Danny's plan did exactly what it was built to do. It covered him well in Cumberland County, where his primary care doctor and the hospital he'd used for twenty years were both in network.

Medicare.gov explains that insurance companies decide whether a plan is available to everyone with Medicare in a state or only in certain counties, and that they may offer several plans in one area with different benefits and costs. Plan availability is local, and non-emergency network coverage away from home varies. Danny's life crossed two service areas.

Which brings me to the question I'd ask before any other. It's the first one this book opened with, and it isn't "which plan is best." It's this: which doctors, hospitals, and pharmacies do you actually need, and where are they standing?

Where your knees actually get looked at, in other words, rather than where your mail arrives.

A person who sees three specialists at one health system fifteen minutes from the house, and leaves the county twice a year for a wedding, is in a very different position from Danny. For that first person, a plan that includes that health system can work out plainly well. Danny's current plan did not include that Florida office in its network. He needs to compare the specific plans available where he

lives and confirm Florida provider access and out-of-network costs with each plan. Medicare's own enrollment page says to ask your doctors and pharmacies whether they're in network with the plan. It also says to check whether the plan will cover you if you live in another state part of the year.

Ask the plan, and then ask the doctor's billing office separately. Directories go stale, and Medicare.gov puts the question to the providers themselves rather than to a printed list.

While we're on geography: neither road travels well abroad. Medicare.gov says Original Medicare generally doesn't cover medical care outside the U.S., though you may be able to buy a Medigap policy that covers emergency care outside the U.S. It says Advantage plans generally don't cover care outside the U.S. either, and that some plans may offer an extra benefit covering emergency and urgently needed services when you travel. If a cruise or a month in Portugal is in your plans, that's a question to ask in writing before you enroll, not after.

The initials after the plan name change the answer

An insurer's name can appear above an HMO and a PPO in the same county. The shared logo does not make the access rules identical. Start with the plan type, then examine the particular product. Medicare.gov has separate official pages for [HMOs](#), [PPOs](#), [Private Fee-for-Service plans](#) and [Special Needs Plans](#). Those pages explain differences a slogan about Advantage cannot cover.

An HMO generally directs routine care through its contracted providers. Medicare's page identifies exceptions for emergency care, out-of-area urgent care and temporary out-of-area dialysis. Some HMO Point-of-Service plans allow specified non-network services with higher cost sharing. The words *Point-of-Service* do not open

every outside office to every service. Have the plan identify what the exception actually covers.

The HMO page also says a primary care doctor is generally required and specialist referrals are required in most cases, with some services excepted. A referral is not the same as authorization: one concerns access through a clinician; the other is a plan's coverage approval. If a specialist's office asks for both, obtaining one has not completed the other. Put that distinction beside the appointment you are trying to arrange, rather than memorizing it as another pair of insurance words.

A PPO has a different non-network structure. Its official page allows covered services from outside providers who participate in Medicare or accept assignment, with higher costs possible, and directs you to check coverage and medical necessity with the plan before receiving such care. It says specialist referrals are not required. That does not mean every recommended service bypasses the plan's authorization conditions or every outside provider agrees to see you.

An outside provider might be eligible under the PPO's rules but unwilling to accept the appointment; a willing provider might offer a service the plan will not cover under the circumstances proposed. Coverage and appointment availability are separate checks. A label promising non-network access cannot book the visit on your behalf.

Private Fee-for-Service, or PFFS, is another Advantage type, not Original Medicare in a private envelope. Medicare.gov says the provider must accept the plan's payment terms, agree to treat you, and not have opted out of Medicare. A participating network can give a more settled provider arrangement, but an outside provider may decide at each visit whether to accept those terms. Yesterday's visit is therefore not sufficient assurance for next month's planned care.

The PFFS page says no primary care selection or specialist referral is required, and Part A/B-covered services do not require prior

approval. Prescription coverage is a separate product check: some PFFS plans include it, while a PFFS plan without it can be paired with a separate drug plan. By contrast, the official HMO and PPO pages say that if you want drug coverage with those types, you must select a version that includes it; you cannot simply add a stand-alone Part D plan to a version without drugs. Have SHIP check an unusual combination before enrollment.

Special Needs Plans, or SNPs, tailor benefits and care coordination to qualifying groups. Medicare.gov identifies dual-eligible, chronic-condition and institutional types. You need Parts A and B, residence in the service area, and the particular plan's qualifying status. These are HMO or PPO plans, so "special needs" is not a substitute for checking which access model applies. All SNPs provide drug coverage, the official page says.

For a reader with both Medicare and Medicaid, an integrated dual-eligible plan may coordinate the two sets of benefits. That is not an instruction to change your Medicaid arrangement based on this paragraph. Ask the plan and state Medicaid office how the proposed product fits the benefits you actually have. For a chronic-condition plan, ask which condition or group of conditions qualifies; for an institutional plan, verify that its clinicians serve the facility where you live. Availability and eligibility are local questions with individual answers.

One approval protection deserves attention before a change during treatment. Medicare.gov describes a transition safeguard on its HMO and PPO pages: the new plan must allow at least 90 days before asking for new prior approval of ongoing treatment. Those pages also describe approval lasting while the authorized treatment remains medically necessary. This does not establish that an old clinician is in the new network or that all bills will match the old plan's terms.

For planned enrollment during a course of care, have the new plan explain how it will handle that specific course and have the treating office check the arrangement. Give them the treatment and relevant approval, not just the diagnosis. The useful answer identifies what continues, who provides it, and any remaining coverage conditions. Do this before a safe, planned switch; a continuity provision should prevent an interruption, not become a vague reassurance after one occurs.

What the extra benefits are and aren't

The dental and the gym membership are real, and I won't pretend otherwise to make a point.

Medicare.gov says Original Medicare covers most medically necessary services and supplies in hospitals, doctors' offices, and other health care facilities, and that it doesn't cover some services, including routine physical exams, eye exams, and most dental care. It says Advantage plans must cover all medically necessary services that Original Medicare covers, and may also offer extra benefits Original Medicare doesn't, like some vision, hearing, and dental services.

Note the word *some*, which Medicare.gov uses and which the television ad does not. A dental benefit carries its own limit and its own list of what's included, and that limit lives in the plan's own documents rather than in any federal rule I can quote you.

So price the extras honestly. A cleaning twice a year is worth real money. A cardiologist in the month you need him is worth more. If both columns genuinely work for your doctors, then extras are a sensible tiebreaker and I'd use them that way.

Prior authorization lands on your calendar

Prior authorization is easy to underestimate because it sounds administrative.

Medicare.gov says an Advantage plan may require its approval first, and that Original Medicare, in most cases, does not. What that buys you is a third party standing between a doctor's recommendation and the calendar, and sometimes that party says no, and then you're appealing instead of healing.

Advantage plans can fit readers whose providers are in network and who value a medical out-of-pocket limit. The trade is a network and, for some services, prior authorization. For some readers, an Advantage plan's yearly out-of-pocket limit is a genuine protection that Original Medicare alone does not provide. It is a yearly ceiling on your cost-sharing for covered Medicare services, while premiums, drug costs, and non-covered care still require separate budgeting. Anybody who describes that trade as one-sided is selling something.

The usual advice here annoys me in both of its flavors. "Advantage plans are a trap" ignores people for whom a hard dollar ceiling is the difference between sleeping and not. "Original Medicare is for people who don't do math" ignores Danny in a parking lot. Neither slogan has met your doctor list.

When the plan says no

A denial letter can sit beside a bill and make both harder to read. Separate them first. An ordinary deductible or coinsurance charge may be correct; a refusal to cover care, pay a claim, or change a disputed amount is a decision you can challenge. Medicare.gov's [Filing an appeal](#) page describes those rights. A grievance concerns service or care quality rather than that coverage decision. If you cannot identify what happened, ask the decision-maker which process fits before sending anything.

Use this decision tree to find the right door:

1. **Covered care is about to end:** Read the hospital's "An Important Message from Medicare about Your Rights" or the other provider's "Notice of Medicare Non-Coverage." Follow its BFCC-QIO instructions for independent fast review. Medicare.gov's fast-appeals page gives the hospital deadline as the planned discharge day; for skilled nursing, home health, comprehensive outpatient rehabilitation and hospice, it gives noon on the day before the listed termination date. Ask for a missing notice immediately. Medicare Advantage also has plan appeal routes with different rules; do not substitute a plan's ordinary process for the fast-review instructions on your service-ending notice.
2. **Original Medicare decided a claim:** Start with the Medicare Summary Notice's appeal instructions. The first level is a redetermination. Use the claim information, destination and deadline printed there rather than sending it to a private insurer that happens to cover another part of your care.
3. **An Advantage plan denied medical coverage or payment:** Request the plan's review using its written decision. Its first review and later independent review are part of the health-plan appeal route. If waiting could seriously harm your health, involve the treating clinician and ask about expedited review. That urgency request is distinct from a hospital discharge appeal.
4. **A drug plan denied a medicine:** Use that plan's written drug-decision instructions. If no coverage decision exists yet, a coverage determination or exception request may be the first step; Chapter 4 explains the restrictions a prescriber may need to address. An appeal challenges a decision already made, not merely an unfavorable shopping estimate.

Across these routes, Medicare.gov describes generally five review levels. Each decision letter explains whether and how you can continue. You need the next permitted step, not five forms completed at once. If a filing date has passed, ask the decision-maker whether a late request can be considered and what evidence it requires; do not assume extra time is available.

The following checklist is the whole notice-and-submission job. Use it once for the decision in front of you:

- Identify the member, service or medicine, date, claim or decision number, and the exact reason given. Read the full notice, including its final instructions page.
- Copy its filing deadline and accepted submission method. Name the person who will file; a provider's offer to help does not establish that it has submitted anything.
- Ask the provider for the evidence that answers the stated reason: an order, missing record, factual correction, or clinical explanation. Keep unrelated paperwork out of the packet.
- State what you want reconsidered, using the required form or portal. List attachments so the reviewer can connect each document to the disputed decision.
- Retain the submitted copy and transmission confirmation, then check receipt without letting that call replace timely filing. Save the next determination with its new instructions.
- If someone acts for you, ask Medicare or the plan what representative authorization is required. Use its approved channel for private medical information.

Suppose, as a worked example, an Advantage notice refuses an outpatient therapy request because the file lacks the clinician's explanation of why skilled therapy is needed. This is an invented paperwork situation, not an account of a successful appeal. The

response should address the missing explanation, not argue that all therapy must be covered.

The reader gives the clinician the actual notice. The office can identify the requested treatment and explain the relevant medical facts. The appeal's short opening asks for reconsideration of that named service and points to the dated clinical statement. The reader submits through the notice's specified route, keeps confirmation, and reads the resulting decision. If the denial actually cites a different reason, this example's response is the wrong one. Evidence must fit the letter you received.

That is enough structure to begin without inventing an insurance argument. SHIP offers free help reading Medicare appeal documents. When the dispute puts essential treatment or substantial costs at stake, seek prompt assistance from the treating provider and, if legal questions require it, a qualified attorney. A counselor cannot extend a deadline by taking the letter home. An appeal does not promise payment or tell you to stop treatment; urgent medical decisions belong with the clinician, and immediate danger calls for emergency care rather than an insurance queue.

Last year's plan was last year's plan

Here's the part that catches people who did everything right once.

Medicare.gov states that each year, insurance companies can decide to join or leave Medicare. It says that if a plan decides to stop participating in Medicare, you'll have to join another Medicare health plan or return to Original Medicare. It lists reasons a plan can disenroll you, including moving outside the plan's service area, and says those situations come with a grace period and a Special Enrollment Period.

So the plan can change its network. It can change what it charges and what it covers. It can leave your county. The decision you made

last fall was a decision about last year's plan, and the plan you're in next January is a different document with the same name on the envelope.

That's why Medicare is an annual chore rather than a one-time choice, and why the fall window exists at all.

The two windows that let you move

Chapter 2 drew the line between the January-through-March General Enrollment Period, for people who haven't got Part B yet, and the fall window everyone hears about on television. Here is that fall window, and its lesser-known cousin.

Medicare's Open Enrollment Period runs October 15 through December 7. Per [Medicare.gov](https://www.medicare.gov), during those weeks you can join, drop, or switch Medicare Advantage plans, with or without drug coverage. You can switch from Original Medicare to an Advantage plan, or from an Advantage plan back to Original Medicare. And if you're in Original Medicare, you can join, drop, or switch a Medicare drug plan. Coverage starts January 1 of the next year, and the plan must receive your request by December 7.

The deadline is December 7, not New Year's Eve. Write it on the calendar in the kitchen.

Then there's the Medicare Advantage Open Enrollment Period, January 1 through March 31, and [Medicare.gov](https://www.medicare.gov) is specific that it's only for people already in an Advantage plan. During it you can switch to a different Advantage plan, with or without drug coverage. Or you can drop the Advantage plan and return to Original Medicare, which also lets you join a separate Medicare drug plan. The new coverage begins on the first day of the month after your request reaches the plan.

There's a related door for people brand new to all of this. If you joined a Medicare Advantage plan during your Initial Enrollment

Period, Medicare.gov says you may switch Advantage plans or return to Original Medicare during the first three months you have both Part A and Part B. During the January 1-March 31 Medicare Advantage Open Enrollment Period, a person already in an Advantage plan may make only one change.

So a person who enrolls in an Advantage plan at 65, uses it in the first weeks, and discovers what Danny discovered, has a door. One door, once, early on. Whether a Medigap policy is still available to that person on the same terms is a separate question with its own rules. That's Chapter 5, and it's why I'd rather you get this right in the fall than fix it in the spring.

One more thing before the worksheet, and it's the place to stop reading and make a phone call. If you have health coverage now through an employer or a union, yours or a spouse's, Medicare.gov says to ask that benefits administrator about their rules before you join a Medicare Advantage plan. Joining one might, in some cases, cost you the employer or union coverage. Medicare.gov adds that if you drop or lose it for yourself, your spouse and dependents may lose it too, and you may not get it back. That is not a risk to evaluate from a book. Put the question to your benefits administrator in writing, and bring the answer to a free SHIP counselor before you enroll in anything.

Where do you actually spend the year

Five questions. Write the answers down, because the last two are the ones that change people's minds.

1. List every doctor, specialist, hospital, and pharmacy you used in the last two years, and write the city next to each. Add anyone you expect to need: an orthopedist for the knee, the oncologist you still see once a year. If the list spans two states or two health systems, say so at the top of the page in capital letters.

2. Count the months. How many nights did you sleep somewhere other than your own house last year, and where? A month with a daughter in Oregon is not a vacation as far as a network is concerned. Write the months and the places.
3. For each plan you're considering, check every name on your list two ways: in the plan's directory, and by calling the provider's billing office to ask whether they're in network with that specific plan for the coming year. Write the date of the call and the name of the person who answered. Then ask Medicare's Plan Finder at [Medicare.gov](https://www.medicare.gov) whether the plan serves your county.
4. Finish this sentence on paper: in a bad year, I could pay out of pocket up to _____ before it damages us. That household limit is the number everything else gets measured against, and it's the one people answer last and feel first.
5. Build a **same-use scenario**, not a worst-case ranking. Choose one plausible medical year, such as an inpatient admission followed by six specialist visits and a course of outpatient therapy, with the same services, settings and prescriptions for every option. Ask the providers and each insurer to price those assumptions for the coverage year. Keep annual premiums on their own line: Part B, then the Advantage premium or the Medigap and separate Part D premiums. On a second line, enter estimated cost sharing for the scenario's covered medical services, after any supplement pays. On a third, enter the drug-only estimate for that same prescription list and pharmacy; do not add premiums or deductibles twice. On a fourth, price the same uncovered-care assumptions, such as dental work, using actual quotes. Record each Advantage plan's in-network limit and any out-of-network or combined limit separately as protections, not predicted bills. Ask which limit applies to the providers in this scenario and which expenses count toward it. For Medigap, price the medical share left by that specific policy

version; for bare Original Medicare, note that the estimate has no annual medical ceiling. Write “cannot estimate” beside any unpriced service or uncertain coverage rather than treating it as zero. Add only the priced components, label the total incomplete if anything is missing, and ask SHIP to check this comparison before a coverage switch.

If question 1 names two states, or question 5 produces a total above the number in question 4, you have a real conflict rather than a simple preference, and that’s exactly the conversation to take to a SHIP counselor, who is free and sells nothing. Medicare.gov keeps the directory at shiphelp.org and lists the agent and broker marketing rules on its own site, which is worth a look before you take anybody’s call.

Danny’s second winter

Danny and I spent an evening at my kitchen table the following October with his list, his two states, and a legal pad. He made a different choice for the next year, and I’m deliberately not telling you which one, because his list isn’t yours and that’s the entire point of this chapter.

Go write your list. Doctors, cities, months. One page, in pencil.

Chapter 4 takes the same honest-arithmetic approach to the drugs you pick up at the counter, where the sticker premium misleads even more reliably than it does here.

Chapter 4: The Premium Is Not the Price

Joan had a pharmacy printout folded in half on our kitchen table. She read the names off it while I entered them into Medicare's Plan Finder: one at a time, with the strength and the number of pills. Then we did it again with the pharmacy we actually used.

The plan with the lowest monthly premium looked like the bargain until the yearly estimates were on the screen. For our list and that pharmacy, another plan came out lower over the year. We hadn't found a trick. We'd finally compared the thing we were going to pay, not just the first number on the page.

That distinction matters more with prescriptions than almost anywhere else in Medicare. Two plans can both offer Part D and still cover different drugs, price the same drug differently, or use different pharmacies for their best rates. A plan name is not a promise about the bottle you pick up.

A list of medicines is not a list of plan prices

Medicare.gov explains that each Medicare drug plan sets its own premium, deductible, copayments or coinsurance, and list of covered drugs, within Medicare's rules. Drug coverage can come from a stand-alone Part D plan or from a Medicare Advantage plan that includes drug coverage. If you have employer, union, or other health benefits, ask the benefits administrator how a change would affect them before you switch anything.

The list of covered drugs is called a formulary. It is specific to the plan and coverage year. One plan may cover the drug you take; another may not. A covered drug can also land in a different cost tier, and the plan may set conditions for coverage. Read the current formulary and the plan's Evidence of Coverage, not last year's printout or a search result with a familiar insurer's name on it.

Check the details against the prescription as it is written: exact drug name, strength, form, and usual quantity. A tablet and a liquid with the same medicine are not interchangeable entries in a price estimate. If your prescription has a generic option, enter the medicine you actually take first. You can ask your prescriber whether an alternative is appropriate, but don't substitute one on your own to make a plan comparison look better.

The pharmacy matters, too. A plan can have preferred pharmacies, standard network pharmacies, and pharmacies outside its network. The price shown can change depending on which one you choose. Mail order may be an option, but do not assume it is cheaper or suits every prescription. Check the pharmacy you would really use, and check another nearby option only if you would actually go there.

Three restrictions that are not prices

A formulary entry can show a medicine as covered and still leave work to do before the pharmacy can fill it under the plan.

Medicare.gov's [Drug plan rules](#) page distinguishes prior authorization, step therapy and quantity limits. Treat those entries as conditions to investigate, not decorations beside a reassuring price.

Prior authorization requires approval under the plan's conditions. The prescriber may need to establish medical necessity or show that the drug is being used for a condition the plan covers. A prescription

order and an authorization approval are different documents. The doctor can intend a medicine for you without the plan having received the information it requires to cover that use.

For shopping purposes, the question is whether your current prescription meets that plan's conditions and what must be submitted if it does not. Ask the prescriber's office to evaluate the clinical requirements, not to promise that a familiar diagnosis will automatically satisfy them. A plan's previous approval under an old product or coverage year should not silently become the assumption behind a new plan estimate.

Step therapy requires trying a specified drug before coverage of another drug under the rule. Medicare.gov explains that an exception can be requested when the prescriber considers the requested medicine necessary, the alternative likely to cause adverse effects, or the alternative less effective. The prescriber supplies the medical explanation. The reader's preference for a bottle is not the same thing as that supporting statement.

If you already tried the proposed first-step drug, tell the prescriber and identify the relevant treatment record. Do not try it again on your own to make the paperwork look tidy. If you have not tried it, the clinical question is whether it is appropriate for you, not whether a plan worksheet becomes cheaper after you type in its name. This book cannot decide that treatment question, and a drug comparison should not decide it accidentally.

A quantity limit concerns the amount covered within a period. The official page uses a plan covering 30 tablets per month as an example, not a limit applying to every prescription. Compare the restriction with the actual directions on your label. One tablet twice a day does not fit the same monthly quantity as one tablet daily. Entering only the drug's name can conceal that difference from the person reviewing your shopping sheet.

The prescriber may request a quantity exception when the plan's limit is not medically appropriate for the patient. That is different from asking a pharmacy to split a fill without knowing how the plan will process it. Put the prescribed amount and the stated limit side by side, then let the plan and prescriber establish the route. Do not stretch doses or ration a medicine while waiting for a price question to be resolved.

These three restrictions can make two identical-looking formulary prices different practical choices. The lower estimate may depend on a rule you have not yet satisfied. On the worksheet, mark that cost conditional until the required coverage question is answered. A verified restriction is more useful than a favorable total built on an untested assumption.

An exception is a clinical request, not a shopping discount

Medicare.gov's [How do drug plans work?](#) page describes formulary exceptions and tiering exceptions. The first can seek coverage for a drug absent from the list or waiver of a rule. The second can seek a lower cost-sharing level for a medicine on a non-preferred tier. The terms matter because they ask for different changes.

The prescriber must provide a supporting medical statement. Ask the plan what request is available for the particular medicine and situation; do not assume that every specialty drug can simply be moved to the cheapest tier. If you are comparing plans before enrollment, an exception you hope to obtain is not existing coverage. Keep the advertised benefit and the requested departure from it on separate lines.

A useful shopping note might say that the medicine is absent and the prescriber is reviewing an exception request. That note identifies uncertainty without predicting its outcome. It would be misleading

to enter the price of a listed alternative while continuing to assume you will receive the original drug. Compare what is actually covered and flag what remains undecided.

Plans can change their formularies during the year under Medicare's guidelines, the official page explains, and must notify you of changes affecting medicines you take. It also notes that coinsurance can rise when the manufacturer's price rises. Fixed copayments and percentage-based coinsurance do not respond identically to a changing price. Read a notice about the particular medicine when it arrives rather than treating last fall's estimate as a contract for every later refill.

If an exception or coverage request is denied, use Chapter 3's appeal tree rather than starting a second procedure from these shopping notes. The denial's instructions determine the next step. Before a denial, the job here is narrower: identify the restriction, have the prescriber assess it, and ask the plan for the correct coverage request.

The first fill may arrive before the card

There is a practical gap between joining a plan and holding its plastic card. Medicare.gov's [Using your drug coverage](#) page explains what to show the pharmacy if the new card has not arrived: the plan's acknowledgment or welcome letter, or its enrollment confirmation number together with the plan name and phone number. An official printable Medicare card may also help the pharmacy locate the coverage record.

Keep the plan's effective date with those items. Evidence that an application was received does not authorize you to treat future coverage as active today. Ask the plan and pharmacy to confirm the coverage they are using for the fill, especially near a changeover. A

rejection caused by missing membership information is not necessarily a clinical denial of the medicine.

Bring photo identification and the drug-plan card when available, plus the Medicare card. For someone with Medicaid or Extra Help, the official page says to bring proof of that assistance too, so the pharmacy can apply the correct drug charges. Chapter 7 handles eligibility and applications; at the counter, this is about showing the benefit already awarded.

If the pharmacy cannot locate the plan information and you pay yourself, save the detailed receipt and contact the plan. Medicare.gov says some spending may be reimbursed or credited toward eligible out-of-pocket costs. It does not promise that every cash purchase will be refunded. Ask which claim process applies before assuming the amount belongs in the covered-drug estimate.

When new drug coverage begins, Medicare.gov's rules page says you may receive a one-time transition fill of a medicine you already take that is absent from the formulary or subject to prior authorization or step therapy. It describes a 30-day supply and tells you to check the particular plan's rules. That possible bridge is not permanent coverage and is not a reason to ignore a restriction during enrollment.

Ask what happens before the next fill is due. The prescriber may need to request an exception, complete authorization, or discuss a clinically suitable change. A temporary supply gives time for that work if it applies; it does not settle it. Keep treatment decisions with the prescriber, especially when an interruption could be harmful.

A pharmacy receipt may belong to a different part

Part D does not own every medicine used by a person with Medicare. The official drug-plan explanation says Part A generally covers drugs

supplied as part of treatment during a covered inpatient hospital or skilled nursing stay. Part B covers a limited set of outpatient drugs under its conditions, including some administered in a clinician's office. The place and method of administration can change the coverage question.

Insulin supplies a concrete example without adding another price chart. Medicare.gov's [insulin page](#) assigns insulin used in a pump covered under Part B's durable-equipment benefit to Part B. Injectable insulin and insulin used with some other pumps may be covered through Part D. Do not change equipment or delivery method for a plan estimate. Ask the treating team and plan which benefit applies to the actual product and device you use.

That distinction matters to a yearly worksheet. A drug paid through a medical benefit should not quietly be priced as a Part D prescription and then counted toward the Part D cap. Keep any medical-benefit drug expense with the appropriate medical estimate. The drug's familiar name does not establish which program pays the particular claim.

Hospital outpatient care creates another awkward boundary. Medicare.gov explains that Part B generally does not cover the self-administered medicines you would normally take yourself while in an emergency department or observation setting. Part D may cover them under certain circumstances, with payment first and a reimbursement claim afterward. Ask the hospital for an itemized drug charge and the drug plan how it handles that claim. Do not assume that a hospital wristband places the whole medicine bill under Part A.

Vaccines also divide between benefits. The official drug-plan page names COVID-19, flu, hepatitis B and pneumococcal vaccines among those covered by Part B, while Part D covers other medically necessary commercially available vaccines under its rules. Its guidance checked in 2026 says adult vaccines recommended by the

Advisory Committee on Immunization Practices have no out-of-pocket charge under Part D. Ask the vaccinating provider which benefit it will bill and whether the service meets the coverage conditions; do not predict a charge merely from the word “shot.”

For a planned service, that benefit check can happen before the appointment. It is not a medical recommendation to get or skip a vaccine. The clinician supplies that advice. The coverage question is which card, provider arrangement and program rules apply to the service recommended for you.

Price the pharmacy you can reach

The [official pharmacy page](#) distinguishes preferred contracted pharmacies from other contracted locations. A preferred pharmacy may charge less than another pharmacy in the same network. “In network” therefore answers a coverage-access question without necessarily identifying the lowest cost.

Mail order can provide up to a three-month supply for some covered medicines, Medicare.gov says. Before counting its estimate, consider whether you can use that arrangement for the prescription and where deliveries would go. Ask about the plan’s actual mail-order terms and refill service, rather than assuming every medicine can be supplied that way. A hypothetical saving at a pharmacy you will not use is not a household saving.

For an outside-network purchase, the official page warns that you may have to pay the full price. It says to retain receipts and ask the plan whether part can be refunded, with out-of-network cost sharing not refundable. Contact the plan about its conditions. Do not carry the preferred-pharmacy estimate onto a receipt from somewhere else and expect the two amounts to match.

When comparing a nearby pharmacy with mail order, hold the drug, quantity and coverage year constant. If the quantities differ,

identify that difference before attributing the whole price gap to the pharmacy. This is a test of the comparison's inputs, not another instruction to choose the lowest visible number.

The number to compare is the whole year

The monthly premium is real. It is also only one line. Medicare.gov's drug-cost page lists premiums, deductibles, copayments and coinsurance as possible costs; the amounts depend on the plan. Use the same prescriptions, quantities and pharmacy for every plan you compare, then inspect the Plan Finder's cost breakdown so you know which pieces its annual estimate includes.

Here is the arithmetic to put on paper:

1. Write the monthly premium for the plan year, then multiply it by 12. If the plan has a separate income-related charge, keep that separate; Chapter 6 covers that subject.
2. Add the Plan Finder's estimated yearly spending for your prescriptions, using its breakdown to identify projected cost-sharing. If the displayed annual total already includes the plan premium, do not add that premium again; if it separates premium and drug spending, combine those lines. Do not add the deductible a second time when it is already reflected in the estimate.
3. Add costs the estimate does not cover, such as a medicine the plan excludes or a purchase you expect to make outside the plan. Ask the plan how that purchase is treated before counting it as covered spending.
4. Write the plan name, coverage year, pharmacy, date you checked, and any assumptions beside the total. Keep the worksheet with the formulary and plan documents you used.

The result is an estimate, not a bill. Your prescriptions can change; the plan's rules can affect when a medicine is covered; and a doctor may prescribe something new midyear. But using one list consistently is a much better comparison than setting a premium from one plan beside a premium from another and calling the smaller one a winner.

If a plan's estimate surprises you, look for the item causing the difference. Is the drug not on the formulary? Is it in a higher tier? Did you select a different pharmacy or quantity? Did the estimate include a deductible? Check the plan's own current documents, then call the plan if the answer remains unclear. Write down the date and the name or reference number for the call.

The cap has a year attached to it

Part D also has an annual out-of-pocket limit for covered drugs. This is not a ceiling on everything you spend for health care, and it is not a ceiling on every dollar you spend at a pharmacy. The plan premium is separate. A drug that is not covered may not count toward the limit. Medicare.gov's cost page explains which eligible spending counts, including certain payments made on your behalf, such as Extra Help.

The current figure is not the figure printed last year. Medicare.gov's Part D cost page lists a \$2,100 out-of-pocket limit for 2026 and a \$2,400 limit for 2027. It also lists the maximum plan deductible as \$615 in 2026 and \$700 in 2027; some plans set a lower deductible or none at all. Those are year-specific federal limits, not a forecast of what any one person's plan will cost. The plan's own premium, formulary and cost-sharing still matter.

After your counted spending on covered Part D drugs reaches the limit for that coverage year, Medicare.gov says you automatically enter catastrophic coverage and pay nothing out of pocket for

covered Part D drugs for the rest of that year. The phrase *covered Part D drugs* is doing important work. A premium remains a premium. A drug the plan does not cover is a different problem, and a medical service is not part of this drug limit.

Medicare.gov's figures can change as the program's annual amounts are set. As I write on October 4, 2026, its page gives both the 2026 and 2027 Part D amounts. Before making a decision, confirm the number for the year your coverage starts at Medicare.gov's Medicare drug coverage cost page. If a later official update changes a figure, use the new official figure, not this paragraph.

The cap is useful for a particular kind of bad year: one with substantial covered prescription costs. It cannot tell you which plan covers your medicines or whether you can use your pharmacy. Nor does the cap make every Part D plan cost the same. Your comparison still begins with the list in your hand.

The penalty follows a gap, not a bad shopping choice

Missing Part D when you first become eligible can have a longer tail than one enrollment season. Medicare.gov says you may owe a late enrollment penalty if, after your Initial Enrollment Period, you go 63 or more days in a row without Part D or other creditable prescription drug coverage. Creditable means coverage expected to pay, on average, at least as much as standard Medicare drug coverage.

The penalty is generally based on the number of full uncovered months and the national base beneficiary premium for the relevant year. Medicare.gov describes the calculation as 1% of that base amount for each uncovered month, rounded to the nearest ten cents and added to your Part D premium. The base amount can change. I am not putting a 2027 penalty dollar figure here; check

Medicare.gov's current penalty calculation for the year it applies to you.

The additional amount normally stays attached to Part D each month you remain enrolled, including after a plan switch.

Medicare.gov says people who qualify for Extra Help do not pay it while receiving that help. The plan tells you whether it has assessed a penalty when you join. If the notice does not match your coverage history, ask the plan how it counted the months and contact Medicare or a SHIP counselor for help reviewing the decision.

Do not decide that another policy is creditable just because it pays for prescriptions. Medicare.gov advises people to look for a notice from their employer or other coverage provider stating whether the drug coverage is creditable. If you are not sure, ask that provider in writing before you drop the coverage. This is especially important for people with retiree, union, VA, or other coordinated benefits; one change can affect benefits beyond the drug plan itself.

Use Plan Finder as a calculator, then verify the result

Medicare's Plan Finder is built for the comparison Joan and I needed. Medicare.gov now lets people review 2027 plan options. For each plan, enter your ZIP code, the year you need, every prescription, the correct dose and quantity, and each pharmacy you are genuinely willing to use. If you have a Medicare account, the site may let you use saved drugs and pharmacies; still check that the saved list is current.

Then compare the plans on the same terms. Start with the yearly drug-cost estimate, not the monthly premium column. Look at the premium and deductible as well, so you understand why the estimate differs. Check the coverage details for every medicine that matters to you and check each pharmacy. Save or print the results with the

date. A result without its year and assumptions is a scrap of paper with a short memory.

Plan Finder gives an estimate, not an exact pharmacy charge or a promise that your prescriptions will stay the same. Before enrolling, review the plan’s current formulary and Evidence of Coverage. If one important medicine is absent or has a restriction you cannot work around, call the plan and ask what options it offers. Your prescriber can discuss clinically suitable alternatives. If you are about to run out of an essential medicine or believe a coverage decision is wrong, contact the prescriber and the plan promptly rather than waiting for the next enrollment season.

If you need help with the comparison, a SHIP counselor can walk through the choices without selling you a plan. Medicare.gov also lists 1-800-MEDICARE. A licensed insurance agent may explain plan terms, but ask who the agent represents and how they are paid; do not let a sales call replace checking the plan’s own documents. If you have complex employer or retiree benefits, talk with that benefits administrator before enrolling or disenrolling.

A worksheet that fits beside the pharmacy printout

Use one copy for each plan. Keep the coverage year the same on every page.

1. Plan name and coverage year:

2. Monthly premium: _____ x 12 = annual premium:

3. Pharmacy selected in Plan Finder:

4. Prescriptions entered, including strength and quantity: _____

5. Plan Finder's estimated annual drug spending:

6. Does that estimate include the annual premium? Yes / No / unclear
7. Important coverage conditions or excluded drugs to verify: _____
8. Likely purchases not included in the covered-drug estimate: _____
9. Estimated annual total, with assumptions written beside it: _____
10. Date checked and plan contact, if you called:

Do not write the annual deductible again as an extra cost if the Plan Finder estimate already includes what you would pay toward it. The deductible is part of the route through the estimate, not automatically a second bill on top of the estimate. If you cannot tell what the total includes, ask the plan or Medicare before relying on your arithmetic.

If income-related Part D charges may apply, keep them on a separate line and use the current figure for the year in question; Chapter 6 explains the lookback and appeal route. If you may qualify for Extra Help, check before ruling it out. Chapter 7 covers where to apply and how it changes drug costs.

Joan still keeps that pharmacy list in the kitchen drawer. The useful part is not the drawer. It's that the list has the names, strengths and quantities, so we don't have to start from memory every fall.

Before you compare plans, make your own list from the labels or your pharmacy's current printout. Put the strength next to each name. Then take that page to Medicare.gov and price the year, not the advertisement.

Chapter 5: Six Months That Don't Come Back Every Fall

The application is open on a laptop, waiting at a question about hospital stays. The applicant has had Part B for several years. Now they want Medigap, and the company wants a medical history before deciding whether to sell it.

This is an illustration, not somebody else's claim file. Suppose the applicant left Original Medicare's unpaid share to savings at first, then decided to buy a policy after treatment made that share harder to manage. Outside a protected buying window, an insurer may use medical underwriting. The application can end with a higher price, fewer choices, or a refusal.

Nothing about being allowed to return to Original Medicare answers that application question. Chapter 3 gave you the switching calendar. Medigap has its own calendar, and a right to make one change does not establish a right to buy the other coverage.

I'd rather you meet that distinction here, with no application pending.

What happens to the unpaid share

Medicare.gov's Medigap pages describe a policy that helps pay your portion of covered Original Medicare bills. You generally need both A and B to buy it. Medicare pays first. The supplement pays according to its benefits, and whatever remains is yours.

It addresses specified deductibles, copayments and coinsurance. Depending on the letter, an item may be paid completely, paid partly

or left to you.

Medicare.gov says Medigap generally excludes long-term care, vision and dental care, hearing aids, glasses and private-duty nursing. If you're imagining help bathing and dressing in a nursing home, don't mistake a supplement's skilled-nursing coinsurance benefit for insurance against that long-term expense. They answer different bills.

Some policies include foreign travel emergency benefits, within limits. Read the conditions before relying on one abroad.

For prescriptions, keep the comparison from Chapter 4; a newly purchased Medigap policy does not add drug coverage. And the supplement belongs on the Original Medicare side of Chapter 3's comparison. Medicare.gov says it cannot pay an Advantage plan's premiums or cost sharing. If you're arranging a return to Original Medicare, confirm that transition separately from the supplement application.

A Medigap policy covers one person, according to Medicare.gov. A couple needs separate policies if both want it, with each person's eligibility, premium and effective date on their own paperwork.

The letter is the benefits, not the company's name

In most states the standardized choices are Plans A, B, C, D, F, G, K, L, M and N. These are Medigap plan names. Plan B is not Medicare Part B; Plan D is not prescription coverage. Somebody allowed both alphabets into the same room, and the rest of us have to label our papers carefully.

According to Medicare.gov's basics page, companies selling the same letter sell the same standardized basic benefits. For that coverage comparison, price is the difference between insurers.

Paying more for a standard Plan G does not turn it into a richer standard Plan G.

Choose the benefits first. Comparing prices for different letters can make less coverage look like a discount on the same thing.

There are qualifications. Medicare.gov lists high-deductible versions of Plans F and G in some states. A high-deductible G and a standard G do not start paying at the same point. Write out the version, not just the letter, and check the current figure for the high deductible on Medicare.gov's comparison page.

Medicare SELECT is another qualification. Medicare.gov says these policies may require particular providers and may have a lower premium. Ask whether a quote is for SELECT before treating it as interchangeable with a standard policy. Chapter 3's provider list still has work to do here.

Medicare.gov explains that Plans C and F are unavailable to people newly eligible for Medicare on or after January 1, 2020. Earlier eligibility can change that answer, including for someone who had Medicare before 65. First eligibility controls, not the day you decided to shop. Ask Medicare or SHIP if your history makes the cutoff unclear.

The letter chart is not an alphabetical ranking. Read the benefits you would buy.

Read down the chart, not across an advertisement

Open Medicare.gov's [Compare Medigap Plan Benefits](#) page. It shows each benefit as a separate row. A full-coverage mark applies to that named benefit; it does not mean the policy covers every medical expense.

Start with the Part A deductible and hospital cost sharing from Chapter 1, then skilled nursing facility coinsurance. Ask what the

candidate pays in each row. A “No” means that policy leaves that row’s cost with you, however generous the rows above look.

Next examine Part B. Standard Plan G leaves its annual deductible to you while covering the coinsurance and excess-charge benefits shown in the federal chart. That describes bills after the deductible, without promising payment for excluded services.

Plan N has a different shape. Medicare.gov’s chart identifies copayments for some office visits and emergency department visits when you aren’t admitted as an inpatient. It does not cover the Part B deductible or the excess-charge row. Read the current copayment amounts on the official page and ask the insurer to show those provisions in the policy summary.

Medicare.gov’s provider-assignment page explains that a non-participating provider may charge above Medicare’s approved amount when not accepting assignment. That extra is an excess charge. Use Chapter 1’s provider check; ask the billing office and SHIP about your exposure and any applicable state restrictions. This is different from a provider opting out of Medicare entirely.

Plans K and L leave percentages of several covered costs to you. The federal chart shows their annual out-of-pocket limits and explains that full payment of covered services for the remaining calendar year follows meeting both the applicable limit and the annual Part B deductible. Check the current figure for each limit. It is not a limit on premiums or on services the policy excludes.

Plan M pays only part of the Part A deductible shown in the chart. Put the unpaid portion into Chapter 3’s worksheet, where a cheaper premium can be weighed against bills you keep.

Mark two or three candidates. If a lower premium persuades you, write a sentence identifying the expense you’re agreeing to keep.

Three states need a different page

Massachusetts, Minnesota and Wisconsin standardize Medigap differently. Medicare.gov names these exceptions on its basics page and provides a comparison for each state.

If you live in one of them, use [Medigap in Massachusetts](#), [Medigap in Minnesota](#), or [Medigap in Wisconsin](#). Those pages, not a Pennsylvania letter comparison, are your starting point.

For example, Medicare.gov's Massachusetts comparison separates hospital deductibles, skilled nursing coinsurance and other benefits under state-specific choices. Take the names from your state's page and ask the company to identify the precise policy and any added coverage in its quote.

State variation also matters beyond these three states. Medicare.gov directs readers to their State Insurance Department for additional buying or switching protections under state law. Don't infer those protections from a neighbor's experience across a state line. Ask which rule applies to your residence, age and proposed change, and have the counselor identify the current state guidance.

Find the date beside MEDICAL

The federal Medigap Open Enrollment Period lasts six months. Medicare.gov says it begins with the first month in which you are both at least 65 and enrolled in Part B. This is a buying protection tied to your own circumstances, not a season printed on every calendar.

During that period an insurer cannot use your medical history to deny you a policy it offers or raise your price because of pre-existing health problems. You still pay the policy's premium. The protection concerns access and health-based pricing; it does not make all letters cost alike or freeze your premium for life.

Count whole months. Someone already 65 whose Part B begins July 1 has a July-through-December window. An October start gives that person October through March. Crossing into a new year doesn't restart the count.

If Part B starts later than 65, the Medigap clock starts when those two conditions first meet. Whether delaying Part B was safe belongs to Chapter 8's employer-coverage check, not to a sales conversation about a supplement.

If you already have Part B through disability before 65, reaching 65 supplies the age condition for this federal window. Before 65, Medicare.gov says federal law generally does not require companies to sell you Medigap; state law may provide rights. Take an under-65 question to your State Insurance Department rather than applying the ordinary age-65 shopping advice early.

There is one more qualification to the protected window. Medicare.gov's buying guidance says coverage can start without a general wait, but allows an exception for coverage related to a pre-existing condition. If a company proposes a pre-existing-condition wait, ask it to count your prior creditable health coverage: at least six continuous months before applying during this window bars that exclusion; shorter creditable coverage reduces it. Ask for the condition, credit and policy effective date in writing. This health-coverage test is not Chapter 4's creditable drug coverage; the two words travel together and mean different things. A right to purchase and payment for a particular condition are distinct questions.

Use the date printed beside MEDICAL on your Medicare card, together with the month you reach 65. If the resulting window is open, don't put the decision off until an October advertisement reminds you about Medicare.

When the six months are behind you

Medical underwriting means the insurer evaluates your health information when deciding whether to accept you and on what terms. Medicare.gov warns that after open enrollment you may face higher prices, fewer options, or denial if you do not meet the company's requirements.

That is the risk in the laptop illustration. Savings can pay a bill today; they cannot create a protected application next year. I'm not telling you to buy a premium you cannot sustain. I'm telling you that waiting is a decision about future access as well as this month's money.

The six-month federal period doesn't repeat each fall. Chapter 3's annual plan-change dates do not require a Medigap company to accept you. Nor does finding a cheaper company's policy mean that company must sell it to you on the same terms.

But a closed window is not automatically a closed case. Medicare.gov describes separate *Medigap protections*: rights to buy specified policies without a health-based refusal in qualifying circumstances. Additional state rights may apply. The policies available and the conditions depend on the particular protection; don't treat the phrase as permission to buy every letter whenever you like.

Medicare.gov's explanation of how Medigap works gives two relevant trial-right examples. If you dropped Medigap to try Medicare Advantage for the first time, there is a single 12-month trial period to return to your former policy if the company still sells it. If it doesn't, certain other policies may be available, subject to the applicable rules.

Separately, if you joined Advantage when first eligible for Part A at 65 and return to Original Medicare within the first year, Medicare.gov describes a right to buy certain Medigap policies sold

in your state. That age-and-first-eligibility condition matters. It isn't a fresh trial right after every Advantage enrollment.

If either example fits, take the enrollment dates and notices to SHIP promptly. For a voluntary trial-right return, ask SHIP about applying as early as 60 days before Advantage coverage ends and no later than 63 days after it ends; keep the disenrollment notice and proof of prior coverage. Confirm the specific right and dates before switching. Returning to Original Medicare inside that first year does not leave the application open indefinitely afterward. Confirm coordinated coverage dates with the insurers before giving up existing coverage.

A health-based refusal, an approaching deadline, or a proposed switch after your protected window is where I'd involve a SHIP counselor and, if needed, a state-licensed insurance agent. The agent can handle company-specific applications; ask which insurers they represent. Your State Insurance Department can clarify state protections. The reason for the call is practical: the right letter is of little use if the application cannot be accepted.

A quote you can keep paying

Chapter 3 already put the supplement premium into your household total. Here, make sure the figure is an official quote for your actual application, not just the estimate on Medicare.gov.

The agency's cost page says premiums depend on the insurer, policy and residence; discounts and underwriting can affect the price. Ask which assumptions and discounts the quote includes. Compare what you'd actually pay.

Ask how the company sets premiums and what can cause increases. Medicare.gov cautions that premiums typically rise each year. Choosing the same benefits from a lower-priced insurer is

sensible; assuming today's price will last for the rest of retirement is not a budget.

For an existing standardized policy, Medicare.gov says health problems do not prevent its automatic renewal. That protection does not stop premium increases, excuse unpaid premiums, or erase the consequences of an untruthful application. Keeping a policy and applying to a different company are different transactions. Don't cancel the first on the strength of an estimate for the second.

The letter-and-date checklist

Use this alongside the official comparison, with one sheet per candidate. In Massachusetts, Minnesota or Wisconsin, substitute your state's policy names for letters.

1. Record your residence, age and Part B start date. Write the beginning and end of your six-month window. If it has passed, have SHIP identify any applicable federal or state protection before you rely on a quote.
2. Copy the exact plan letter or state-specific name. Label standard, high-deductible or SELECT explicitly. Confirm first-eligibility restrictions if you are considering C or F.
3. Mark the official chart's hospital deductible, hospital cost-sharing and skilled nursing rows. Write each amount or portion the candidate leaves to you.
4. Mark the Part B deductible, coinsurance and excess-charge rows. For N, check the office and emergency-visit copayments. For K or L, record the current limit and its conditions.
5. Check foreign emergency benefits if you need them. Keep excluded expenses, such as dental work and long-term care, outside the policy's protection when you budget.

6. Obtain official quotes from more than one company selling the same choice. Record monthly premium, discounts, pricing assumptions and the date. Ask about future increases.
7. Ask about medical underwriting, any pre-existing-condition waiting period and the effective date. If a protection applies, confirm the evidence the company requires and its deadline.
8. Put the quote and residual expenses into Chapter 3's budget. Keep prescription coverage on Chapter 4's separate sheet. For a couple, do the calculation for each person.
9. Read and retain the company's policy summary. Resolve unanswered questions with SHIP, the insurer or your State Insurance Department before changing coverage. Keep written acceptance and coverage dates with your records.

Start with the card, not the brochures. Copy the Part B date onto the checklist tonight. If those six months are running, give the page a place where it won't disappear under the mail.

Chapter 6: The Tax Return Behind the Premium

The notice says your Medicare premium is higher. The tax return on the desk is from two years ago, and your pay has changed since then. Those two facts can both be true at once.

The extra amount is called the income-related monthly adjustment amount, shortened to IRMAA. The name is longer than the idea: if your income on the tax record Social Security uses is above a published line, you pay an additional monthly amount for Part B, Part D, or both, depending on the coverage you have. It is not a bill for a service you used. It is not the Part D plan's own premium. And a lower income today does not, by itself, change the return already used to set this year's amount.

That last part is what makes the notice feel personal. The system is looking backward while your household is living forward.

Which income, and which year

Social Security's [Medicare premiums page](#) says it uses the most recent federal tax return the IRS provides. Its measure is modified adjusted gross income, or MAGI: adjusted gross income plus tax-exempt interest. This is not simply the amount of money that came into your bank account, and it is not the number at the top of a pay stub.

For the 2026 Medicare premium year, SSA says it generally uses the return filed in 2025 for tax year 2024. In some cases, SSA may have the 2023 return instead. The two-year lookback is a useful rule

of thumb, not a reason to ignore the year named in your own notice. Find the tax year SSA actually used before trying to explain the calculation.

If SSA used 2023 for your 2026 premium but you filed a 2024 return, or did not have to file one for 2024, call SSA or visit a local Social Security office and ask it to update the tax year used.

A household can cross an income line for a reason that no longer exists. A person may have worked a full year before retiring. A spouse may have died. An asset sale or another taxable event may have raised the return's MAGI. The premium notice does not know that the paycheck stopped in January; it uses the tax information sent to SSA. The right response depends on whether the old return is accurate and whether a qualifying event has since lowered income.

IRMAA is individual to the Medicare premium decision, but the income bands depend on tax filing status. A joint return is compared with joint thresholds. A person filing as an individual is compared with the individual schedule. Married people who filed separately and lived with their spouse at any time during the tax year have a separate, much narrower schedule. Do not use that special schedule just because the words "married filing separately" appear on a return; confirm the living-situation rule on SSA's current page.

The 2026 brackets, in one place

The following are the 2026 amounts published by Social Security. The income ranges use MAGI from the tax return SSA applies. Part B's listed figure is the monthly surcharge added to the standard Part B premium. Part D's is the separate monthly income-related charge added to the premium of your drug coverage. The plan's own Part D premium still varies by plan.

For people filing an individual return, and married couples filing jointly, the schedule is:

- MAGI of \$109,000 or less for an individual, or \$218,000 or less jointly: no IRMAA surcharge.
- More than \$109,000 through \$137,000 for an individual, or more than \$218,000 through \$274,000 jointly: Part B surcharge of \$81.20 per month; Part D surcharge of \$14.50 per month.
- More than \$137,000 through \$171,000 for an individual, or more than \$274,000 through \$342,000 jointly: Part B surcharge of \$202.90 per month; Part D surcharge of \$37.50 per month.
- More than \$171,000 through \$205,000 for an individual, or more than \$342,000 through \$410,000 jointly: Part B surcharge of \$324.60 per month; Part D surcharge of \$60.40 per month.
- More than \$205,000 but less than \$500,000 for an individual, or more than \$410,000 but less than \$750,000 jointly: Part B surcharge of \$446.30 per month; Part D surcharge of \$83.30 per month.
- \$500,000 or more for an individual, or \$750,000 or more jointly: Part B surcharge of \$487.00 per month; Part D surcharge of \$91.00 per month.

SSA also publishes a distinct schedule for certain married people who file separate returns. Whether it applies depends on whether the spouses shared a home during that tax year:

- MAGI of \$109,000 or less: no IRMAA surcharge.
- More than \$109,000 but less than \$391,000: add \$446.30 monthly to Part B and \$83.30 to Part D.
- \$391,000 or more: the monthly additions are \$487.90 for Part B and \$91.00 for Part D.

These are the surcharges, not the complete premium. For Part B, add the applicable surcharge to the standard Part B premium for 2026 shown in Chapter 1. If you have Part D coverage, add its surcharge to

that plan's premium; the surcharge is not a substitute for the plan premium. If you do not have one of those kinds of coverage, SSA says you pay an IRMAA only for the benefit you do have. For example, a Part B enrollee without Part D does not acquire a Part D charge merely because the tax return falls in a higher band. If your notice adds a Part D adjustment but you have no Medicare drug coverage, call 1-800-MEDICARE to correct the coverage record CMS sends SSA.

The top line on the separate-return schedule is not identical to the top individual or joint line. SSA lists a slightly different Part B add-on there for 2026. Don't copy one household's notice onto another person's worksheet and assume the filing status matches.

SSA may collect the additional amount from a Social Security payment. Its page says that if the charge is greater than the monthly benefit, or if you do not receive a monthly Social Security payment, you may receive a separate bill from CMS or the Railroad Retirement Board. The collection method does not change what the notice is saying. It is still worth checking the underlying income year and filing status before you accept the result as correct.

The brackets and surcharges are not permanent household thresholds. SSA publishes the schedule for each premium year, and the amounts can change. Use its current official table when planning for 2027 or any later year. Don't carry a 2026 number forward because it happens to be printed in this chapter.

A lower income can matter, but the route has rules

Retirement is the life change people ask about first. Stopping work can qualify for a new decision when it lowers the income SSA uses. The word "retired" by itself is not a pass through a lower premium.

SSA needs a qualifying event and information showing the income change.

Its [Form SSA-44](#), titled *Medicare Income-Related Monthly Adjustment Amount—Life-Changing Event*, is the route to ask SSA to make a new determination after certain events. SSA lists these life-changing events:

- You married, divorced, or had a marriage annulled, or your spouse died.
- You or your spouse stopped working or reduced work hours.
- You or your spouse lost income-producing property because of a disaster or another event beyond your control.
- Your or your spouse's employer pension plan stopped or was reorganized.
- You or your spouse received a settlement from an employer because the employer closed, went bankrupt, or reorganized.

The event must connect to lower income under SSA's rules. A planned retirement may fit when work stops and income falls; deciding to retire while income stays above the applicable line does not automatically change the calculation. A market decline, a difficult medical year, or the fact that a premium is uncomfortable is not, by itself, one of the listed events. Read the current form instructions before you send anything.

Form SSA-44 asks what happened and requests information and evidence for SSA to review the change. Follow its current instructions for estimating income and documenting the event. Keep a copy of the completed form, the notice, supporting papers, and the date you sent them. If a letter or employer record establishes the stop-work date, include it as directed by the form. Do not mail your only copy of a document you may need again.

This is a request for a new determination because circumstances changed; it is different from saying SSA used the wrong information. If SSA relied on an incorrect tax return or MAGI, SSA's page directs you to correct the information with the IRS when the IRS data itself is wrong. If you amended a return and the amendment changes MAGI, send SSA the amended return and the IRS acknowledgment according to its instructions. If you disagree with SSA's decision for another reason, the appeal route is separate: SSA identifies an online appeal and Form SSA-561-U2, Request for Reconsideration. Check the notice and SSA's current instructions for the right path and deadline.

A tax professional can help if you cannot confidently estimate MAGI after a complicated sale, pension event, amended return, or employer settlement. That professional can help with the tax calculation; SSA decides how the IRMAA rules apply. For questions about the Medicare premium notice itself, use Social Security's official number or local office, and keep the notice in front of you when you call. A SHIP counselor can also help you understand the Medicare side without selling a plan.

The surcharge is separate from the plan decision

IRMAA does not change a drug plan's formulary or make one insurer's plan fit your prescriptions better. Chapter 4's comparison still needs the same medicines, strengths, quantities, and pharmacy. Put the Part D surcharge on its own line beside the plan premium so it does not disappear from the household estimate or get mistaken for a plan benefit.

The same separation helps with Part B. Chapter 1 explains the standard Part B premium and deductible; the IRMAA amount is an added premium charge determined by income. It does not replace

the deductible or coinsurance. If you have both Part B and Part D, you can see two income-related amounts. If you have only one, SSA says its adjustment applies only to the coverage you have.

The surcharge also does not decide when to claim Social Security. Claiming age is a separate decision; IRMAA is based on the tax information and premium-year rules SSA applies. The larger claiming strategy belongs in *Social Security, Claimed On Purpose*, not in this chapter's arithmetic.

If you are thinking about a tax move because of a Medicare premium, pause before acting. A change to income can affect taxes and other parts of a household's finances, and a Medicare chart alone cannot tell you whether a conversion, sale, or withdrawal is wise. Ask a tax professional to model the consequences before you make an irreversible transaction. This book can help you read the premium notice; it cannot replace tax advice.

A notice-and-life-change checklist

Set the notice beside the tax return rather than trying to reconstruct the story from memory. Use this checklist for each person who received a determination.

1. Write down the premium year, the Part B amount, any Part D amount, and the tax year SSA says it used. Compare the notice with SSA's schedule for that exact premium year.
2. Check filing status and the MAGI line. Remember SSA's definition: adjusted gross income plus tax-exempt interest. For an MFS return, check the household-status qualification in SSA's current instructions before selecting a bracket.
3. Decide which problem you have. If the IRS return or SSA's income information is wrong, follow the correction or appeal instructions. If the tax information was accurate but a qualifying

event later lowered income, read Form SSA-44. For a Part D surcharge despite having no drug-plan enrollment, ask CMS to correct its coverage record at 1-800-MEDICARE; SSA receives that record from CMS.

4. If the notice uses tax year 2023 for 2026 premiums, contact Social Security when you filed for 2024 or had no 2024 filing requirement; ask the agency to update its record.
5. Match the event to SSA's list. Record its date and how it changed income; gather the evidence the current form requests. Stopping work can be relevant, but retirement alone does not settle the request.
6. Complete the form using its current instructions and keep the full submission, evidence, and mailing or upload confirmation. Note any response date SSA gives you.
7. If you cannot calculate the income estimate, ask a tax professional. If the notice or Medicare coverage question is unclear, call SSA or a SHIP counselor rather than relying on a sales pitch.
8. Put the next premium-year table check on your calendar. Check SSA's current IRMAA page and Medicare.gov before using any 2026 amount for a later year.

Take out the notice now and circle the tax year printed on it. That date is the first thing to verify.

Chapter 7: The Application Worth Sending

Marcus's message arrived on a Tuesday, while the library table was still empty. His mother was paying for prescriptions and had decided the help programs were for people with no income at all. He had helped her look at the official rules, then sit down with an application. She qualified for Extra Help. Her drug costs dropped.

That is the whole story, and it is enough. No miracle, no exact savings figure to promise you. A program helped because somebody checked the rules instead of guessing from the name.

The two programs in this chapter answer different bills. Extra Help lowers certain costs for Medicare Part D prescription coverage. Medicare Savings Programs, run by states, can pay some Part A and Part B costs. A person may qualify for one or both. The income and resource tests change. For MSPs, states may use more generous rules than the federal baseline. I won't give you a number to carry into another year. Medicare.gov's current pages show the limits for the year they apply to; check the current figure before deciding you are over it.

This is not the high-income surcharge from Chapter 6 turned upside down. It is a separate eligibility check, with its own agencies and paperwork.

Extra Help follows the drug bill

Medicare.gov calls Extra Help a program for people with limited income and resources who need help with Part D costs. Depending

on the current rules and your coverage, it can help with a plan premium, deductible, coinsurance, and copayments for covered prescription drugs. It can also protect you from the Part D late-enrollment penalty while you receive the help.

The details that matter at the counter still depend on your drug plan and the prescription. Chapter 4's formulary and pharmacy checks remain useful: Extra Help does not make an uncovered medicine covered, and it does not tell you which plan fits your list. Check the current [Medicare.gov](https://www.medicare.gov) drug-cost page and the plan's information for the year you are choosing. If the amount at the pharmacy doesn't match your Extra Help notice, call the plan or 1-800-MEDICARE while you still have time to sort it out before a refill runs out.

Some people get Extra Help automatically. [Medicare.gov](https://www.medicare.gov) says that includes people with full-benefit Medicaid, people whose state pays their Part B premium through a Medicare Savings Program, and people receiving Supplemental Security Income from Social Security. If one of those applies, watch for the official notice. It may tell you how much you pay and, if you do not already have Part D coverage, which drug plan Medicare has selected for you.

Do not file the notice under “junk” just because you didn't fill out an Extra Help application. Read the plan name, effective date and instructions. If a plan was selected for you, compare it against your current prescriptions and pharmacy. Chapter 4 explains that comparison; this chapter's point is to make sure the help and the coverage are both actually in place.

If you are not automatically enrolled, you can apply through Social Security. The application and current instructions are at [SSA.gov](https://www.ssa.gov)'s Extra Help page. You can apply online or ask Social Security about other application methods. Keep a copy of what you send and any confirmation. For help understanding the questions, a

SHIP counselor can walk through the Medicare side without selling you a plan.

The application asks about financial circumstances under the rules in force. Don't rely on a neighbor's old limit, an article without a year, or an online calculator that doesn't name its source. Medicare.gov says Extra Help limits can go up each year. Its page also explains which income and resource figures apply, including separate treatment for Alaska and Hawaii. Check the current year and the rules for your state before you rule yourself out.

That is one reason applications get abandoned in the kitchen drawer. A person sees a limit, counts every dollar in the wrong way, and decides the answer is settled. The official application instructions are the place to check what counts. If a pension, property, or unusual account makes the answer hard to calculate, ask Social Security or SHIP how to proceed rather than inventing a formula.

The state programs pay different parts of the bill

Medicare Savings Programs, or MSPs, are administered by state agencies. Medicare.gov says they can help with Part A or Part B premiums; some categories can also help with deductibles, coinsurance and copayments for Medicare-covered services. The state decides which program fits after you apply. A Medicare card does not tell you whether the state has finished that decision.

There are four federal program names on Medicare.gov. They are not four labels for the same benefit.

- The Qualified Medicare Beneficiary program, or QMB, can pay the Part B premium, Medicare cost-sharing for covered Part A and Part B services, and a Part A premium if you owe one. Medicare.gov says providers may not bill a QMB enrollee for

Medicare-covered deductibles, coinsurance or copayments. Keep proof of QMB status with your Medicare card and show it when you receive care. The protection concerns covered Medicare services; it is not a promise that every service or bill is covered.

- The Specified Low-Income Medicare Beneficiary program, or SLMB, helps pay the Part B premium. Medicare.gov says you need both Part A and Part B to qualify under this category.
- The Qualifying Individual program, or QI, also helps pay the Part B premium for people who meet its rules. Medicare.gov says QI applicants must apply each year, and states approve applications on a first-come, first-served basis, with priority for people who received QI the prior year. Check the current state process and funding before assuming last year's approval automatically continues.
- The Qualified Disabled and Working Individuals program, or QDWI, can pay a Part A premium for a narrower group: people with a disability who are working and lost premium-free Part A and certain Social Security disability benefits after returning to work. If that description might fit, ask the state to review the situation; don't try to decide from the program name alone.

The exact eligibility requirements and monthly income and resource limits are on Medicare.gov's [Medicare Savings Programs page](#). They change over time. The federal page lists separate limits by program and household size, with different figures for Alaska and Hawaii. It also warns that some states may allow eligibility above the federal figures or disregard certain kinds or amounts of income and resources. The federal number is a starting point, not a ruling on your state's application.

A person can have too much for one category and still be eligible for another. The benefits are not identical, either. QMB is the broadest of these descriptions for Medicare cost-sharing; SLMB and

QI focus on the Part B premium; QDWI addresses the Part A premium in its specific working-and-disability circumstances. Let the state determine the fit rather than trying to squeeze your household into a label before applying.

Why the word “resources” stops people

Income and resources are separate parts of these tests. The program may not treat every dollar or asset the way you expect. The rules can differ by program, state and year. A balance on a bank statement, a home, a vehicle, a retirement account, and money coming in monthly are not interchangeable entries on a worksheet. Don’t assume all count; don’t assume none count.

The helpful question is not “Am I poor enough?” It is “Which current rule does my state use for this program, and what documents does it need from me?” That’s a more answerable question.

Medicare.gov links to state Medicaid contacts for the application and local criteria. If you live in Alaska or Hawaii, use the relevant state-specific figures. If the state page doesn’t explain an item you own or receive, ask the state office before excluding yourself.

It may be tempting to use a broad income number from a tax return as a substitute for the program’s test. Resist that shortcut. The IRMAA calculation in Chapter 6 uses a specific tax measure and lookback; it is not the application rule for these programs. The Social Security Administration and state Medicaid agency can tell you which documents their process requires. A tax return may be one document, but it is not a universal stand-in for the full eligibility decision.

A denial is not a reason to guess at a new number or quietly give up. Read the reason, the date and the instructions for asking the state to review or appeal. If your income changed, your household changed, or you think an item was counted incorrectly, ask what

evidence the agency will accept and what deadline applies. A SHIP counselor can help identify the Medicare piece; a state eligibility worker is the person to ask about the state's calculation. If a complicated property or income question affects several benefits, a qualified benefits counselor or attorney may be appropriate because the consequences can reach beyond Medicare.

Put both applications on the same page

Medicare.gov says you can apply for Extra Help and a Medicare Savings Program at the same time. Its current guidance says Social Security can send information to the state to start an MSP application unless you tell it not to on the Extra Help application. That handoff is a convenience, not proof that the state has all it needs or has approved you. Watch for a request from the state and answer it by the stated deadline.

For Extra Help, Social Security is the application route. For an MSP, your state Medicaid office makes the eligibility decision. One phone call to the wrong office can leave you with a referral and no application, so be clear about which program you are asking about. If you want to check both, say so.

A person who qualifies for an MSP that pays the Part B premium may also qualify automatically for Extra Help under Medicare's rules. Even then, read the notices. Automatic eligibility for assistance does not mean you can ignore the Part D plan name, the pharmacy rules, or the start date. Some people are automatically placed in a drug plan if they don't already have Part D coverage. Check the plan against your medicines and ask Medicare or SHIP about a change if something does not fit.

Keep the documents together: the Extra Help application confirmation, any request from the state, the approval or denial notice, and the plan's drug-cost information. Write down the name

of the person you spoke with, the date, and what they asked you to send. Paperwork has a talent for becoming three separate stories in three separate envelopes.

The limits belong to the current year

The 2026 and 2027 information on the official sites is not a permanent eligibility chart. Medicare.gov says MSP income and resource limits rise each year and notes that states can be more generous. It says Extra Help limits may also increase. Those pages are better than a chart clipped from an old benefits packet, but even a federal page needs its year checked before you apply.

Do not use last year's denial as proof you cannot qualify this year. Your income or resources may have changed; the limits may have changed; and the state may count an item differently than you expect. Medicare.gov specifically encourages people who are unsure about MSP eligibility to apply. If your circumstances sit close to a limit, that is a reason to get the current rules, not a reason to round your way out of the program.

If you are helping a parent, ask permission before collecting financial papers or submitting anything. Let the person read the notices and keep a copy. A relative can help organize a file without taking over the decision. Marcus helped his mother get through the process; the application was hers, and the result came from the program's review.

The eligibility-check worksheet

Use one copy for each person. Fill it from official sources, not memory. Leave a blank if you cannot verify an answer; that blank is a question for the agency.

1. Write the date, state of residence, household size and the person applying. Note whether the question is about Extra Help, an MSP, or both.
2. Check whether full-benefit Medicaid, SSI, or an MSP paying the Part B premium may already qualify the person for Extra Help automatically. Look for a notice and confirm the status with the agency if it is unclear.
3. Open Medicare.gov's Extra Help page. Record the eligibility-limit year, the income and resource figures for the applicant's household, and the source date. If it is not the current year, stop and find the updated information.
4. Open Medicare.gov's MSP page and copy the current limits for each category that might fit. Mark the relevant state link and check for state rules that disregard income or resources or use higher limits.
5. List the program-specific help: Part D plan costs and penalty protection for Extra Help; Part A or Part B premiums and, for some MSP categories, Medicare cost-sharing. Do not write "all medical bills" in the margin.
6. Apply for Extra Help through Social Security. Contact the state Medicaid office to apply for an MSP, even if you also submitted an Extra Help application.
7. Keep copies, submission dates and confirmation numbers. Calendar any document deadline or renewal date in the decision notice, especially an annual QI application.
8. When a decision arrives, record the effective date, program name, what it pays, what remains yours, and any next review date. If it is denied, follow the notice's review instructions and deadline.
9. If the numbers or documents are confusing, ask the agency that decides that program. Use SHIP for Medicare questions and a

qualified benefits professional when a difficult income or asset question may affect several programs.

A rule in a book can point you toward the door. It cannot decide what the state counts in your case. Open the current Medicare.gov limits today, then make the call or start the application before the envelope slides back under the stack.

Chapter 8: The Paycheck, the Health Plan, and Part B

The payroll form is already filled out. A box is checked beside the Health Savings Account, and the employer adds money each payday. Then a Medicare card arrives in the mail, and suddenly two reasonable choices seem to collide: keep the job plan, or sign up for Medicare.

This is not one decision. It is three: who pays first, whether you need Part B now, and what Medicare enrollment would do to HSA contributions. The answers depend on the kind of coverage you have and whose current work provides it. The number of people employed by the company matters, too.

Don't let a general rule made for someone else's workplace settle this for you. Ask the benefits administrator to explain your specific plan in writing, and check the current Medicare and IRS guidance before you sign up or keep contributing to an HSA.

Start with who pays first

Medicare and an employer group health plan can both cover you, but one generally pays before the other. That order affects whether the job plan will pay its share if you don't enroll in Part B.

CMS's Medicare Secondary Payer guidance says that for someone 65 or older with a group plan tied to their own or a spouse's active job, employer size generally determines the payer order. At an employer with at least 20 employees, the group plan generally pays

first and Medicare second. Below that threshold, Medicare generally pays first and the group plan second.

That difference is practical, not merely bookkeeping. When Medicare is primary and you haven't enrolled in Part B, the employer plan may calculate its payment as though Medicare had paid first. You could be left with costs you thought the job plan would cover. With a qualifying plan from a larger employer, the group coverage may be primary, which can make delaying Part B an option for some people. It is still not an automatic recommendation.

CMS describes this as a high-level summary of payer rules. Multi-employer arrangements and unusual work situations can complicate the count, and other Medicare eligibility circumstances can have different coordination rules. Ask the benefits office which employer's plan covers you, how the employer counts for Medicare coordination, and which insurer is primary. If the answer is unclear, contact Medicare or the Benefits Coordination & Recovery Center before relying on it.

Ask for the plan's current Summary of Benefits and the section explaining coordination with Medicare. A card in your wallet proves that you are enrolled in the employer plan; it does not tell you whether the plan expects Medicare to pay first. If the benefits office gives you a verbal answer, send a short email repeating what you understood and ask them to correct it if needed. Keep that reply with the plan papers. You want an answer you can show the claims office later, not a memory of a phone call made between meetings.

The plan's label won't answer the question. A benefits administrator can tell you how the plan is designed to coordinate with Medicare; Medicare can confirm enrollment rules. Ask both sides. Keep names, dates and the answer in writing.

Four questions before you choose

First, ask, “Who pays first if I have Medicare and this plan at the same time?” Get the answer for your exact coverage. When Medicare is primary, ask what the plan will pay if you delay Part B. When the group plan is primary, ask whether its benefits remain the same after you become Medicare-eligible. The plan document and benefits office should settle the details, not a colleague’s story from another company.

The second question is whether you are still covered because you or your spouse is actively working. Coverage that continues after a job ends is a different thing. Chapter 2 explains the Special Enrollment Period for Part B: the working-coverage window and its deadline are tied to current employment and group coverage ending. Use that chapter for the complete timing and paperwork. Here, the working decision is simpler: don’t mistake a retiree plan, COBRA, or an individual policy for active-employee coverage that preserves that same enrollment route.

The third question is what happens to the rest of the household if you change coverage. A spouse may be on your employer plan, or you may be covered through a spouse who is still working. Ask whether enrolling in Medicare or leaving the group plan changes the spouse’s eligibility, premium, or enrollment rights. Ask what happens to a dependent still on the policy. Get the effective dates. A choice that works for you can create a gap or a higher bill for somebody else if the household rules are different from what you assumed.

The fourth question is whether an HSA changes the calendar. This is the part that catches people who believe they can sign up for Part A and keep contributing to the account because they’re still at work. The IRS says Medicare enrollment makes the contribution limit zero for the affected months, including months covered retroactively. Part A can start retroactively when some people enroll after 65, so the month you apply may not be the first month that matters.

Part A and the HSA calendar

An HSA is not the same as a regular savings account with a tax-free label. You can generally keep and spend money already in the account for qualified expenses, but Medicare enrollment blocks new contributions for the covered months. That includes money from your employer as well as payroll deductions.

The IRS's current Publication 969 explains that HSA eligibility ends with Medicare enrollment and that retroactive coverage counts. Medicare.gov says premium-free Part A can, in some cases, begin as much as six months before the date a person signs up, but no earlier than the month they turned 65. That makes the enrollment date and the coverage-effective date two different dates to check. If you have an HSA, do not treat the application date as a safe cutoff by itself.

For example, imagine someone still working at 67, covered by a high-deductible employer plan, and contributing to an HSA. They plan to apply for Social Security benefits in November and then learn that Medicare enrollment may begin retroactively. Medicare.gov advises you and your employer to stop HSA contributions six months before you retire or apply for Social Security or Railroad Retirement Board benefits. If Medicare backdates Part A coverage, contributions made for covered months may exceed the allowed amount. The surprise does not come from a new contribution in November; it can reach back into earlier payroll deposits. This is a made-up calendar illustration, not a claim about a particular worker's eligibility or tax result.

Before applying for Part A, ask Social Security what start date would apply in your case and ask your HSA administrator or tax professional how to handle contributions around that date. If you plan to enroll, coordinate with payroll and the employer so contributions stop when they should. If contributions have already been made for a month that later turns out to be covered by Medicare, don't guess at a fix or assume the employer can simply

reverse it. Ask a tax professional how to review the contributions, correction options and tax reporting under the current IRS instructions.

The HSA balance stays. The payroll deposits may have to stop months before you apply for Medicare; get both dates on paper before you give payroll an instruction. The question is whether the employer coverage and HSA contributions are valuable enough to keep delaying Medicare, given the payer order, the coverage you need, and the deadline that will apply when employment ends.

If you already receive Social Security benefits when you approach 65, check any Medicare enrollment notice and the effective date on the card rather than assuming you are still outside Medicare. Enrollment can happen automatically in some circumstances. An HSA payroll contribution that continues after Medicare enrollment can create a tax problem even when nobody meant to make a mistake.

A spouse's job can be your bridge, but check the bridge

People often say, "I'm covered through my spouse, so I can wait." That can be right, but the useful details are whose employment provides the group coverage, whether that spouse is still working, and how large the employer is for Medicare's payer rules. CMS includes a spouse's current employment in its working-aged coordination guidance. If the coverage comes through a retired spouse, it is not the same active-employment situation.

Ask the employer to put four things in a letter or benefits email: whether the policy is based on current employment; the employer-size category used for Medicare coordination; whether the group plan pays before or after Medicare; and what event ends the coverage for you. Ask whether your spouse's continued employment maintains

your coverage if you personally retire. Then ask what happens if your spouse retires first. A family plan can have two retirement dates and two Medicare decisions, even if the insurance card has one policy name.

Do not assume that a plan which pays first will stay unchanged once you enroll in Medicare. Ask about the plan's coordination rules, how claims are submitted, whether the plan expects you to have Part A or Part B, and what it pays if you do not enroll. Medicare.gov explains that if an employer or retiree plan is secondary, you may need Part B before it pays. Confirm the answer for the plan you actually have.

If you are leaving work, put the last day of employment, the last day of group coverage, and the requested Medicare start date on the same page. Chapter 2 owns the full eight-month Special Enrollment Period explanation; its clock is not a reason to wait until the last minute. Gather proof of employer coverage and ask the benefits office to complete the employer section of Form CMS-L564 if required for your application. Medicare.gov's ready-to-sign-up instructions describe the forms and where to send them. If you have railroad employment, follow the Railroad Retirement Board's instructions instead of assuming the Social Security process is identical.

This is also the moment to bring in help. If the employer's written answer conflicts with what the insurer tells you, call Medicare or the Benefits Coordination & Recovery Center and ask how to verify who pays first. A SHIP counselor can help you compare the Medicare side without selling a plan. A tax professional should review HSA contributions or retroactive coverage because excess contributions can affect your return. If an employer plan is changing your spouse's coverage, ask the benefits administrator before you submit a Medicare application or drop anyone from the policy.

The decision list

Write the answers down for yourself and any spouse or dependent on the plan. Leave a blank where the employer has not confirmed something; a blank is a question, not a yes.

1. **Whose active work supplies the insurance?** Write your employer or your spouse's employer, the coverage end date, and whether this is active-employee coverage rather than retiree or continuation coverage.
2. **Who pays first?** Record the employer-size category and the primary payer confirmed by the benefits administrator. If the workplace is small or uses a multi-employer arrangement, ask specifically how it coordinates with Medicare. If there is disagreement, verify with CMS or Medicare.
3. **What would Part B change?** Ask for the employee and spouse premiums, deductibles, cost-sharing, provider access, and any effect on dependents. Ask for the employer's current written notice stating whether its prescription coverage is creditable for Part D; keep it and compare its end date with the Part D enrollment instructions in Chapter 4. Medicare.gov warns that a gap of 63 days or more without Part D or creditable drug coverage may cause a penalty. Compare the whole household coverage, not just the premium deducted from your pay.
4. **Is there an HSA?** Set a stop date for deposits from both sources no later than six months before planned retirement or an application for Social Security or RRB benefits; that is Medicare.gov's guidance. Ask Social Security or RRB which Part A effective date applies, then have a tax professional review the tax-year contribution limit before you instruct payroll. Record the Part A and Part B dates, including any retroactive Part A month. Include employer money in the review; it counts toward HSA contributions.

Then make one phone call with the list in front of you. Ask the benefits office to confirm its answers by email. Ask Medicare or Social Security about your enrollment window and effective date, not whether your employer plan is “good.” If an HSA is involved, ask your tax professional to check the calendar before you apply. The right next step may be enrollment now, or it may be a carefully documented delay while you keep working. It depends on the coverage and dates you actually have.

Take the benefits letter and your HSA statement out tonight. Mark the expected retirement or Social Security/RRB application date on the calendar and identify the six-month HSA cutoff Medicare.gov specifies. Before making a payroll change, verify the Part A start date with Social Security or RRB and have a tax professional check contributions for the year.

Chapter 9: The Numbers to Check Every Fall

The pencil is beside the open book, and the official page is open on the screen. That is the right time to find a number that has changed: before you use last year's figure to choose this year's coverage.

This is a reference chapter, not another tour of Parts A, B, or D. Most dollar figures below were printed earlier; the 2026 Part D national base premium is added for penalty checks. They are a dated snapshot, not a promise about what you will pay in another year. Use the blanks to keep the year, the updated amount, and the page you checked together. A number without its year is a number waiting to mislead you.

Three places to check

Start at Medicare.gov's [Medicare costs page](#). It separates Part A and Part B from Advantage, Part D, and Medigap. For Part A and Part B, find the current-year premium and deductible sections. Read the smaller cost-sharing entries too; do not assume the daily amounts stayed put because the main premium looks familiar.

For prescription coverage, use Medicare.gov's [Part D costs page](#). Check the cap for covered Part D drugs, the maximum deductible a plan may set, and the base beneficiary premium if you are reviewing the late-enrollment-penalty calculation. A plan may set a lower deductible or none, so the federal maximum is not a quote for a particular plan.

For IRMAA, open Social Security's [Medicare premiums page](#). Choose the schedule for the premium year you are checking, then match the filing status and income range on your notice. The added Part B amount and the separate Part D amount belong beside the right band; do not copy a neighbor's line or use a prior-year schedule because the tax return looks similar.

CMS and SSA release yearly cost information in the fall, but release dates vary. Mid-to-late October is a pattern, not a promised deadline, and different figures can appear on their official pages at different times. CMS's [Newsroom](#) carries official announcements and fact sheets; Medicare.gov and SSA.gov are where you should confirm the amount that applies to your coverage.

I checked these pages on October 4, 2026. Medicare.gov's general costs page and SSA's IRMAA page displayed 2026 amounts. Medicare.gov's Part D page already displayed the 2027 cap and deductible maximum: \$2,400 (2027) and \$700 (2027). Those are the 2027 Part D figures available on the check date, not estimates. Reopen the pages before acting; a later official update takes precedence over this dated note.

The pencil update list

The dollar-update lines start with dated amounts; items 3 and 8 separately identify rules to verify rather than annual price resets. Fill the current-year box from the official page, and record the date you checked it. If the source does not yet show the year you need, leave the box blank and check again before enrolling or relying on a cost estimate. Do not carry a prior-year number forward just to complete the page.

If you keep this page for next year's review, write a new amount in a dated column rather than erasing the old one. Note what changed: a premium, deductible, daily cost, or income-related addition. Keep

a saved official page or printout with the entry, and mark an unavailable figure “not yet shown” rather than leaving it ambiguous. That small trail lets you compare years without mistaking an estimate for an official amount.

1. **Part A monthly premium:** \$0 for most people (2026); buy-in premium \$311 or \$565 per month (2026). Current amount: _____. *Official source and date checked:* _____.
2. **Part A deductible:** \$1,736 per benefit period (2026). Current amount: _____. *Official source and date checked:* _____.
3. **Part A inpatient hospital daily amounts:** \$0 for days 1-60 after the deductible, \$434 per day for days 61-90, and \$868 per day for days 91-150 while using lifetime reserve days (2026). Current daily dollar schedule: _____. *Separately verify the lifetime-reserve rule: 60 days total during your lifetime, not a fresh allotment each year. Rule source and date checked:* _____.
4. **Part A skilled-nursing daily amounts:** Days 1-20: \$0 daily after any applicable Part A deductible; no second deductible if it was paid during a hospital stay in the same benefit period. Days 21-100: \$217 daily (2026); all costs after day 100. Current schedule: _____. *Official source and date checked:* _____.
5. **Part A hospice cost-sharing (2026):** covered care, \$0; qualifying home pain-or-symptom medicine, no more than \$5 each; inpatient respite, 5% of the approved charge. Current schedule: _____. *Official source and date checked:* _____.
6. **Standard Part B monthly premium:** \$202.90 (2026). Current amount: _____. *Official source and date checked:* _____.
7. **Part B yearly deductible:** \$283 (2026). Current amount: _____. *Official source and date checked:* _____.

8. **Part B cost-sharing rule verification, not an annual dollar update:** Medicare.gov's general rule is 20% after the deductible for a covered service when the provider accepts the Medicare-approved amount; verify its application to your service. Current rule and service-specific exceptions: _____. *Official source and date checked:* _____.
9. **Part B examples from Chapter 1:** \$0 for covered clinical laboratory services, \$0 for covered home health services, and 20% of the Medicare-approved amount for durable medical equipment (2026 cost page). Current amounts or percentages: _____. *Official source and date checked:* _____.
10. **Part D yearly spending ceiling for eligible prescriptions:** \$2,100 (2026); published 2027 limit: \$2,400. Counted out-of-pocket spending includes certain payments made on your behalf; it is not limited to what you personally paid. Later-year limit: _____. *Official source and date checked:* _____.
11. **Part D maximum plan deductible:** \$615 (2026); published 2027 maximum: \$700. Later-year maximum: _____. *Official source and date checked:* _____.
12. **Part D national base beneficiary premium used in the late-penalty calculation:** \$38.99 (2026). Current amount: _____. *Official source and date checked:* _____.
13. **IRMAA standard schedule, first 2026 band:** individual MAGI at or below \$109,000 (2026), or joint MAGI at or below \$218,000 (2026); no added Part B or Part D amount in that band. Current thresholds and additions: _____. *SSA source and date checked:* _____.
14. **IRMAA standard schedule, second 2026 band:** individual MAGI over \$109,000 through \$137,000 (2026), or joint MAGI over \$218,000 through \$274,000 (2026); Part B addition \$81.20 per month (2026), Part D addition \$14.50 per month (2026).

Current thresholds and additions: _____. *SSA source and date checked:* _____.

15. **IRMAA standard schedule, third 2026 band:** individual MAGI over \$137,000 through \$171,000 (2026), or joint MAGI over \$274,000 through \$342,000 (2026); Part B addition \$202.90 per month (2026), Part D addition \$37.50 per month (2026). Current thresholds and additions: _____. *SSA source and date checked:* _____.
16. **IRMAA standard schedule, fourth 2026 band:** individual MAGI over \$171,000 through \$205,000 (2026), or joint MAGI over \$342,000 through \$410,000 (2026); Part B addition \$324.60 per month (2026), Part D addition \$60.40 per month (2026). Current thresholds and additions: _____. *SSA source and date checked:* _____.
17. **IRMAA standard schedule, fifth 2026 band:** individual MAGI over \$205,000 and below \$500,000 (2026), or joint MAGI over \$410,000 and below \$750,000 (2026); Part B addition \$446.30 per month (2026), Part D addition \$83.30 per month (2026). Current thresholds and additions: _____. *SSA source and date checked:* _____.
18. **IRMAA standard schedule, sixth 2026 band:** individual MAGI at or above \$500,000 (2026), or joint MAGI at or above \$750,000 (2026); Part B addition \$487.00 per month (2026), Part D addition \$91.00 per month (2026). Current thresholds and additions: _____. *SSA source and date checked:* _____.
19. **IRMAA married-filing-separately schedule, first 2026 band:** MAGI at or below \$109,000 (2026); no added Part B or Part D amount. Applies if you filed separately and shared a home with your spouse at any point in SSA's tax year; otherwise ask SSA which schedule fits. Current threshold and additions: _____. *SSA source and date checked:* _____.

20. IRMAA married-filing-separately schedule, second

2026 band: MAGI over \$109,000 and below \$391,000 (2026); Part B addition \$446.30 per month (2026), Part D addition \$83.30 per month (2026). Current thresholds and additions: _____. *SSA source and date checked:* _____.

21. IRMAA married-filing-separately schedule, third 2026

band: MAGI at or above \$391,000 (2026); Part B addition \$487.90 per month (2026), Part D addition \$91.00 per month (2026). Current thresholds and additions: _____. *SSA source and date checked:* _____.

The Part D 2027 amounts shown above are already published as of October 4, 2026. Do not treat that as proof that every other 2027 figure has been released. For Part A and Part B premiums and deductibles, and SSA's 2027 IRMAA bands, write down what the official page says on the day you check it. If the page still shows 2026, the honest entry is "not yet shown" plus the date; revisit the page later. Never fill an empty space with a guess.

A 2027 renewal, checked on paper

For example, suppose a reader has a 2027 renewal notice and a prescription list. Enter the published 2027 Part D limits, use that year's Plan Finder with the actual medicines and pharmacy, and mark unpublished Part A/B or IRMAA figures "not yet shown" with the check date.

Keep the Plan Finder estimate beside the plan's Evidence of Coverage, not instead of it. Confirm the same plan year, medicines and pharmacy in both, then record the plan name and date checked. A maximum federal deductible is a ceiling for what a plan may set, not the amount every plan charges. If a missing federal number

could change the choice, ask Medicare or a SHIP counselor what can be confirmed and set a date to revisit the official page.

Keep the source page or a saved copy with the worksheet. When you return to check an amount, compare its label and year with the blank you are filling; a fresh page can still display a prior-year figure. If a plan document and Plan Finder do not appear to agree, confirm the plan name and coverage year before comparing the drug list again. Ask the plan to explain a plan-specific cost; ask Medicare or a SHIP counselor about a federal cost or enrollment choice. Write down the answer and the date, so next fall you can tell a changed rule from a different plan estimate.

When a covered prescription costs more at the counter than the month can bear

The heating bill and a large prescription can arrive in the same week. The voluntary Medicare Prescription Payment Plan lets a Part D enrollee, including someone with Advantage drug coverage, pay eligible prescription costs through monthly plan bills rather than at pickup. Medicare.gov's [Before using this payment option](#) explains that participation has no fee or interest and does not lower the total owed. Your plan premium remains separate.

The public page describes the monthly calculation this way: **this month's eligible prescription costs plus the unpaid balance from the previous month, divided by the months remaining in the calendar year.** New fills change the balance, so the payment need not stay fixed. Use the plan's billing estimate and actual statement for the amount due; the formula is not permission to send a self-calculated payment.

One dated example is enough to see the scale. Medicare.gov's [2026 April example](#) combines a regular prescription costing \$4 with a one-time prescription costing \$613: \$617 of out-of-pocket drug

costs in April 2026. The person elects participation before those fills. Spreading that April expense alone across April through December gives a rough average of \$68.56 for each of nine months in this 2026 illustration. It is not the official installment schedule: recurring refills add costs, and the plan recalculates its bills. Compare the plan's expected April payment with the cash you would otherwise need at pickup, not with a promise of nine identical bills.

Use one enrollment-and-exit checklist:

1. Before the large fill, request participation through your health or drug plan. Record its confirmation, effective timing, first expected bill and due date. Do not assume the pharmacy has received an election the plan has not approved.
2. Compare the schedule with the household's available cash, keeping premiums outside it. Joining after September leaves fewer months and may offer little timing relief. If the total is unaffordable, check Chapter 7's assistance programs rather than treating installments as a discount.
3. Check each statement against the fills and carried balance. Medicare.gov directs billing-error grievances to the process in your Member Handbook or Evidence of Coverage. Its [Using this payment option](#) page says to pay the regular plan premium first to protect drug coverage.
4. To leave, contact the plan. New eligible costs return to pharmacy payment; the existing balance can be paid at once or billed monthly. Changing health or drug plans ends the old participation, so elect again with the new plan if wanted. The previous plan's balance still belongs to that plan.
5. If a payment is missed, use the date in the reminder. Failure to pay by that date removes you from the payment feature, not from the underlying health or drug plan; the balance remains due without interest or fees.

Drug coverage belongs to Chapter 4: use its formulary, restriction and pharmacy guidance for that question. If the plan denied coverage, follow the appeal instructions in its written decision; Chapter 3's 'When the plan says no' section explains the appeal routes.

The pages and dated example were checked October 4, 2026. Verify the later coverage year's program instructions with your plan before using this reference; no 2027 installment amount is promised here.

Put the check date beside the amount

Write the premium year and the date you opened SSA's schedule above the number you copied; for every other entry, record its coverage year, official source and check date. Check available 2027 plan documents now, verify pending agency amounts when published, and do not let the October 15-December 7 plan-change window pass while waiting for a missing figure. Ask a SHIP counselor to review a choice affected by a pending amount.

If the official pages disagree with your notice

For an unexpected Part B or Part D premium, first identify the premium year, the coverage you have, and any income-related amount on the notice. Then compare those lines with the current CMS or SSA page. A plan's own Part D premium is not the same entry as the Part D IRMAA addition. If the amount still does not make sense, call the agency named on the notice and ask which year and data it used. Keep the notice, your worksheet and the call date together.

If an IRMAA notice appears to use the wrong tax information or a year that does not fit, return to Chapter 6's routes: a data correction,

a request tied to a qualifying income-changing event, or an appeal are different matters. SSA decides the IRMAA determination. A tax professional can help when the tax record or a complicated transaction needs review; a SHIP counselor can help with the Medicare coverage side. Bring the notice and the relevant official schedule. Do not rely on an agent's estimate to resolve a Social Security determination.

The book carries 2026 amounts because those were the verified published values used for the Part A, Part B and IRMAA discussions. Its 2027 title names the enrollment season, not permission to invent 2027 figures. Part D is the exception in this dated check: Medicare.gov had published its 2027 cap and maximum deductible by October 4, 2026. As other official numbers become available, the page that publishes them, rather than a printed prediction, gets the last word.

Put the notice beside the official schedule. Circle the year on each before you copy a number.

The Date on the Calendar

The first thing I would do with this book is close it.

Not because the subject is finished. Because the next useful thing is not another page. It is a date written where you will see it.

Medicare decisions have a way of making ordinary people feel they ought to become experts before they can make one sensible choice. You do not. You need a clear question, the current official page, and the patience to stop when the answer belongs to your own file. Worry will not tell you whether the notice applies; the dated notice and the agency that issued it might.

A date, not a season of dread

Medicare.gov lists October 15 through December 7 as the yearly Open Enrollment period for reviewing and changing certain coverage. Mark the October 15, 2026 start, or verify the next applicable start if you read this later; keep the dated answer behind your choice in the folder beside that calendar.

Let a good decision be good enough

No one can know every future health expense, every change in a household, or every adjustment a program will make. That is not a reason to freeze. It is a reason to make the best decision the available evidence supports, preserve the documents behind it, and return when something material changes or the next annual review comes around.

You've checked what you can. Keep the answer you got, and put the pencil down.